

Med-Surg

26 April 2026 22:38

[Med-Surg > Musculoskeletal > Rheumatic Fever] High - VERIFIED

Q2. An 8-year-old girl comes to the clinic complaining of sudden swelling and pain in the knee with no apparent reason. When taking the initial history, which of the following is most important to ask about?

- A) Decreased urinary output and flank pain
 - B) Fever over 103°F over the last 2-3 weeks
 - C) Rash covering the palms and soles
 - D) History of headache, malaise, and sore throat
- Answer: D) History of headache, malaise, and sore throat

Explanation:

Correct - D: Unexplained migratory polyarthritides + recent strep throat = Jones Criteria for Acute Rheumatic Fever. Ask about preceding GABHS infection 2-4 weeks ago.

Incorrect - A: Suggests post-streptococcal GN, not ARF arthritis.

Incorrect - B: Fever is minor criteria, but sore throat history is major clue for etiology.

Incorrect - C: Palms/soles rash = Kawasaki/Syphilis, not ARF.

Trick: Students pick fever B, but "most important to ask" = etiology i.e. sore throat.

Source: NORCET-2023 | Ghai Pediatrics

Tags: #PYQ #HighYield #Pediatric #Cardiology #JonesCriteria #TrickQ

[Med-Surg > Cardiology > Heart Failure] Easy - VERIFIED

Q3. While inspecting a cardiovascular client, the nurse notes that the patient needs to sit upright to breathe comfortably. This behavior is most indicative of which condition?

- A) Pericarditis
- B) Anxiety
- C) Congestive Heart Failure
- D) Angina

Answer: C) Congestive Heart Failure

Explanation:

Correct - C: Orthopnea = breathlessness on lying flat, relieved by sitting up. Patho: In CHF, lying down increases venous return → pulmonary congestion. Sitting reduces preload.

Incorrect - A: Pericarditis = pain relieved by leaning forward, not breathing issue.

Incorrect - B: Anxiety = hyperventilation, no relation to position.

Incorrect - D: Angina = exertional chest pain, not positional dyspnea.

Trick: Orthopnea vs PND vs Trepopnea - all CHF signs.

Source: NORCET-2023 | Brunner & Suddarth

Tags: #PYQ #HighYield #Cardiology #Assessment #Definition

[Med-Surg > Emergency > Shock] High - VERIFIED

Q5. When differentiating between shock states, which clinical sign is characteristically seen in neurogenic shock but NOT in hypovolemic shock?

- A) Skin is warm and dry
- B) Presence of tachycardia
- C) Capillary refill time is less than 2 seconds
- D) Skin is cool and clammy

Answer: A) Skin is warm and dry

Explanation:

Correct - A: Neurogenic shock = loss of sympathetic tone → massive vasodilation → warm, dry, pink skin. Also bradycardia due to unopposed vagal tone.

Incorrect - B: Tachycardia in hypovolemic, but neurogenic has BRADYCARDIA.

Incorrect - D: Cool/clammy = hypovolemic shock due to vasoconstriction.

Trick: Only shock with warm skin + bradycardia = Neurogenic. All others cold + tachy.

Source: NORCET-2023 | ATLS Guidelines

Tags: #PYQ #HighYield #Emergency #Shock #NCLEX #TrickQ

[Med-Surg > Neuro > Spinal Cord Injury] High - VERIFIED

Q6. A patient with a C4 spinal cord injury is preparing for discharge. Because this client is at high risk for autonomic dysreflexia, which primary symptom should the nurse instruct the patient to monitor for?

- A) Dizziness and tachycardia
- B) Pallor and lightheadedness
- C) Severe headache and facial flushing
- D) Pallor and itching of the face and neck

Answer: C) Severe headache and facial flushing

Explanation:

Correct - C: Autonomic Dysreflexia = T6 or above injury + noxious stimulus below level → massive HTN → pounding headache, flushing/sweating ABOVE injury, pale/cool BELOW. Medical emergency.

Incorrect - A/B: These are hypotension signs, AD is hypertensive crisis.

Trick: AD = "A" for Above injury symptoms. Headache is #1 complaint.

Source: NORCET-2023 | Lewis Med-Surg

Tags: #PYQ #HighYield #Neuro #SCI #Emergency #PatientEducation

[Med-Surg > Dermatology > Infections] Easy - VERIFIED

Q9. Honey-colored crust lesions appearing on the face, particularly around the lips and nose, are a classic characteristic of which skin infection?

- A) Impetigo
- B) Measles
- C) Chickenpox
- D) Rubella

Answer: A) Impetigo

Explanation:

Correct - A: Impetigo = GABHS or Staph aureus. Vesicles rupture → honey-colored crust. Highly contagious. Perioral/perinasal common in kids.

Incorrect - B: Measles = Koplik spots + maculopapular rash.

Incorrect - C: Chickenpox = dew drops on rose petal vesicles, not crusts.

Incorrect - D: Rubella = pink macular rash, "3-day measles".

Trick: "Honey" = Impetigo. "Dew drop" = Chickenpox.

Source: NORCET-2023 | Ghai Pediatrics

Tags: #PYQ #HighYield #Pediatric #Dermatology #Identification

[Med-Surg > Renal > ARF] High - VERIFIED

Q18. Which diuretic is strictly contraindicated and should be avoided in a patient suffering from Acute Renal Failure (ARF)?

- A) Furosemide
- B) Spironolactone
- C) Mannitol
- D) Hydrochlorothiazide

Answer: B) Spironolactone

Explanation:

Correct - B: Spironolactone = K⁺ sparing. ARF me already hyperkalemia hoti hai due to low GFR. K⁺ sparing doge toh K⁺ = 7-8 mEq/L → cardiac arrest.

Incorrect - A: Furosemide = loop diuretic, K⁺ waste karta. ARF me hyperkalemia treat karne ke liye dete hain.

Incorrect - C: Mannitol osmotic, used in increased ICP, caution in ARF but not absolute contraindicated.

Trick: "ARF = Already Raised K+" → Never give K+ sparing. Give K+ wasting.

Source: NORCET-2023 | KDOQI Guidelines

Tags: #PYQ #HighYield #MedSurg #Renal #Pharma #NCLEX #TrickQ

[Med-Surg > Renal > Urine Output] Medium - VERIFIED

Q19. Polyuria (excessive urination) is a defining clinical manifestation seen in all of the following conditions EXCEPT:

- A) Diabetes Mellitus
- B) Diabetes Insipidus
- C) Pyelonephritis
- D) Glomerulonephritis

Answer: D) Glomerulonephritis

Explanation:

Correct - D: Glomerulonephritis = $GFR \downarrow \downarrow$ → oliguria/anuria, not polyuria. Fluid retention, edema, HTN hota hai.

Incorrect - A: DM = osmotic diuresis due to glucose → polyuria.

Incorrect - B: DI = ADH deficiency → polyuria 5-20L/day.

Incorrect - C: Pyelonephritis = infection, irritation → frequency + polyuria.

Trick: "Nephritis = No urine". "Diabetes = Diuresis".

Source: NORCET-2023 | Brunner & Suddarth

Tags: #PYQ #HighYield #MedSurg #Renal #Conceptual #TrickQ

[Med-Surg > Dermatology > Fungal Infections] Easy - VERIFIED

Q23. What is the correct medical term for a superficial fungal infection affecting the general body surface (often presenting as a ring-shaped rash)?

- A) Tinea capitis
- B) Tinea cruris
- C) Tinea pedis
- D) Tinea corporis

Answer: D) Tinea corporis

Explanation:

Correct - D: Tinea corporis = Ringworm of body. "Corpus" = body. Annular, scaly, pruritic lesion with central clearing.

Incorrect - A: Capitis = scalp/head. Children me common, hair loss.

Incorrect - B: Cruris = groin/jock itch.

Incorrect - C: Pedis = feet/athlete's foot.

Trick: "Corpus = Core/Body". Capitis = Cap/head, Cruris = Crotch, Pedis = Ped/feet.

Source: NORCET-2023 | Harrison Medicine

Tags: #PYQ #HighYield #MedSurg #Dermatology #Infection #Terminology

[Med-Surg > Infectious > Typhoid] Medium - VERIFIED

Q26. Which diagnostic test is standardly utilized for confirming Enteric (Typhoid) fever during the second week of the illness?

- A) Blood culture
- B) Widal test
- C) Urine culture
- D) Stool culture

Answer: B) Widal test

Explanation:

Correct - B: Widal test = detects O & H antibodies against Salmonella typhi. Antibodies appear end of 1st week, peak in 2nd week. 4-fold rise in titer diagnostic.

Incorrect - A: Blood culture = GOLD STANDARD but best in 1st week when bacteremia high.

Incorrect - C/D: Stool/Urine culture positive in 3rd-4th week during carrier state.

Trick: "Week 1 Blood, Week 2 Widal, Week 3 Stool". B-W-S sequence.

Source: NORCET-2023 | Park PSM

Tags: #PYQ #HighYield #MedSurg #Infectious #Diagnostics #Microbiology

[Med-Surg > Pharma > Cardiac Drugs] High - VERIFIED

Q27. Which serum electrolyte must be closely monitored in a patient receiving Digoxin to prevent life-threatening drug toxicity?

- A) Sodium
- B) Calcium
- C) Magnesium
- D) Potassium

Answer: D) Potassium

Explanation:

Correct - D: Digoxin inhibits Na-K-ATPase pump. Hypokalemia → less K⁺ competition → more digoxin binds → toxicity. S/S: Nausea, vomiting, yellow vision, arrhythmia. K⁺ level 3.5-5.0 maintain karo.

Incorrect - A/B/C: Hyponatremia, hypercalcemia, hypomagnesemia also increase toxicity risk but K⁺ is MOST critical.

Trick: "Low K⁺ = Lots of Digoxin binds = Toxic". Always check K⁺ before digoxin.

Source: NORCET-2023 | Lippincott Pharma

Tags: #PYQ #HighYield #MedSurg #Pharma #Cardiac #Toxicity #NCLEX #TrickQ

[Med-Surg > Neuro > Hydrocephalus] High - VERIFIED

Q28. What is the most common early clinical sign of a Ventriculoperitoneal (VP) shunt malfunction or blockage in an infant?

- A) High pitched cry
- B) Bulging fontanelles
- C) Poor feeding
- D) Sunken eyes

Answer: C) Poor feeding

Explanation:

Correct - C: Early ↑ICP in infant = subtle behavioral changes: poor feeding, vomiting, lethargy, irritability. Fontanelles bulge later.

Incorrect - A/B: High-pitched cry + bulging fontanelle = LATE signs of severe ↑ICP.

Incorrect - D: Sunken eyes = dehydration, not ↑ICP.

Trick: "Feed First Fails" in ↑ICP. Late signs = Crying + Bulging.

Source: NORCET-2023 | Wong's Pediatric

Tags: #PYQ #HighYield #MedSurg #Neuro #Pediatric #Emergency #Assessment

[Med-Surg > Burns > Fluid Resuscitation] High - VERIFIED

Q29. What is the standard Parkland formula used to calculate the 24-hour fluid resuscitation requirement for a severe burn patient?

- A) 2 mL × body weight (kg) × % TBSA
- B) 3 mL × body weight (kg) × % TBSA
- C) 4 mL × body weight (kg) × % TBSA
- D) 5 mL × body weight (kg) × % TBSA

Answer: C) 4 mL × body weight (kg) × % TBSA

Explanation:

Correct - C: Parkland = 4ml x kg x %TBSA burn of RL. 1/2 in first 8 hours from time of burn, 1/2 in next 16 hours. For adults.

Incorrect - A/B: 2-3ml used for children or modified Parkland, but standard exam answer = 4ml.

Trick: "Park in 4 Wheeler" = 4ml. Half in 8 hours, half in 16 hours.

Source: NORCET-2023 | ATLS + Schwartz Surgery

Tags: #PYQ #HighYield #MedSurg #Burns #Calculation #Formula #Emergency

[Med-Surg > Renal > Dialysis] High - VERIFIED

Q33. A patient receiving peritoneal dialysis develops abdominal distention, severe abdominal pain, and redness around the catheter site. What complication do these signs indicate?

- A) Liver cirrhosis
- B) Ascites
- C) Peritonitis
- D) Gastritis

Answer: C) Peritonitis

Explanation:

Correct - C: Peritonitis = #1 complication of PD. S/S: Cloudy effluent, fever, abdominal pain/rigidity, rebound tenderness, catheter site redness/pus. Medical emergency.

Incorrect - A/B: Cirrhosis/Ascites = chronic, painless distention. No acute inflammation/redness.

Incorrect - D: Gastritis = epigastric pain, no catheter site redness.

Trick: "PD + Pain + Pus + Redness = Peritonitis". Cloudy dialysate = diagnostic.

Source: NORCET-2023 | Lewis Med-Surg

Tags: #PYQ #HighYield #MedSurg #Renal #Dialysis #Infection #Emergency

[Med-Surg > Pharma > Electrolytes] High - VERIFIED

Q37. Which of the following medications is NOT used in the emergency management of hyperkalemia?

- A) Calcium gluconate
- B) Insulin and Glucose
- C) Sodium bicarbonate
- D) Spironolactone

Answer: D) Spironolactone

Explanation:

Correct - D: Spironolactone = K⁺ sparing diuretic. Hyperkalemia me doge toh K⁺ aur badhega → cardiac arrest. ABSOLUTE CONTRAINDICATED.

Incorrect - A: Ca gluconate = cardiac membrane stabilizer. First drug in hyperkalemia with ECG changes. Protects heart.

Incorrect - B: Insulin + D25 = shifts K⁺ into cells. Fast action 30min.

Incorrect - C: NaHCO₃ = if acidosis present, shifts K⁺ into cells.

Trick: "C BIG K Drop" = Ca gluconate, Beta agonist, Insulin, Glucose, Kayexalate, Dialysis.

Spironolactone = K⁺ BADhata.

Source: NORCET-2023 | Harrison Medicine

Tags: #PYQ #HighYield #MedSurg #Pharma #Emergency #Electrolytes #NCLEX #TrickQ

[Med-Surg > Cardiology > ECG] High - VERIFIED

Q38. What is the classic electrocardiogram (ECG) finding associated with severe hyperkalemia?

- A) Prominent U waves
- B) ST segment depression
- C) Tall peaked T waves
- D) Widened PR interval

Answer: C) Tall peaked T waves

Explanation:

Correct - C: Hyperkalemia ECG progression: 1) Peaked T waves >5.5 mEq, 2) Prolonged PR + QRS widening >6.5, 3) Sine wave >7, 4) VF/Asystole >8.

Incorrect - A/B: U waves + ST depression = HYPOKALEMIA, not hyper.

Incorrect - D: PR widens later, but PEAKED T = first sign.

Trick: "High K⁺ = High T". "Low K⁺ = Low T + U wave". T = Tall in Hyper, T = flat in Hypo.

Source: NORCET-2023 | Dale Dubin ECG

Tags: #PYQ #HighYield #MedSurg #Cardiology #ECG #Electrolytes #Emergency

[Med-Surg > Trauma > ATLS] High - VERIFIED

Q39. What is the absolute immediate priority management for a severely injured trauma patient presenting in the emergency department?

- A) Control hemorrhage
- B) Stabilize the cervical spine and manage the airway
- C) Assess neurological status
- D) Administer IV fluids

Answer: B) Stabilize the cervical spine and manage the airway

Explanation:

Correct - B: ATLS ABCDE: A = Airway with C-spine protection, B = Breathing, C = Circulation, D = Disability, E = Exposure. Airway FIRST always. No airway = death in 3-5 min.

Incorrect - A: Hemorrhage control is C, comes after A & B.

Incorrect - C/D: Neuro + IV fluids later. Airway blocked toh IV ka kya fayda.

Trick: "A before C". Airway > Circulation. Exception: Massive exsanguination me C-A-B-C.

Source: NORCET-2023 | ATLS 10th Edition

Tags: #PYQ #HighYield #MedSurg #Trauma #Emergency #ATLS #Priority #NCLEX

[Med-Surg > Infectious > Meningitis] High - VERIFIED

Q40. Which cerebrospinal fluid (CSF) analysis finding is highly characteristic of Tuberculous (TB) meningitis?

- A) Normal protein and decreased glucose
- B) Decreased protein and increased glucose
- C) Increased protein, decreased glucose, and increased lymphocytes
- D) Normal protein, normal glucose, increased neutrophils

Answer: C) Increased protein, decreased glucose, and increased lymphocytes

Explanation:

Correct - C: TB Meningitis = ↑Protein 100-500mg, ↓Glucose <40mg, ↑Lymphocytes 100-500.

Chronic infection. Cobweb coagulum on standing.

Incorrect - A/B: Normal protein nahi hota TB me. Glucose ↓ hota, ↑ nahi.

Incorrect - D: Normal protein/glucose + ↑Neutrophils = Bacterial meningitis, not TB.

Trick: "TB = Three Big: ↑Protein, ↓Glucose, ↑Lymph". Bacterial = ↑Neutrophils. Viral = Normal glucose.

Source: NORCET-2023 | Harrison Medicine

Tags: #PYQ #HighYield #MedSurg #Infectious #Neuro #Diagnostics #CSF

[Med-Surg > Trauma > Hemorrhage Control] High - VERIFIED

Q43. A trauma patient presents with a crush injury to the chest and an actively bleeding open femur fracture. What is the immediate nursing priority regarding the limb?

- A) Internal fixation of the femur
- B) Apply a chest tube
- C) Immediately control the external bleeding
- D) Administer pain medication

Answer: C) Immediately control the external bleeding

Explanation:

Correct - C: ATLS: Airway → Breathing → Circulation. Active exsanguinating hemorrhage from femoral artery = Circulation priority. Direct pressure → Tourniquet if needed. Prevent hypovolemic shock.

Incorrect - A: Fixation is OR procedure, not immediate.

Incorrect - B: Chest tube if tension pneumo, but bleeding control first if massive.

Incorrect - D: Pain management after life-saving.

Trick: "C-A-B-C" if massive bleeding: Circulation first, then Airway. Femoral bleed = death in minutes.

Source: NORCET-2023 | ATLS 10th Edition

Tags: #PYQ #HighYield #MedSurg #Trauma #Emergency #Priority #Hemorrhage

[Med-Surg > Pharma > Cardiac Drugs] Easy - VERIFIED

Q44. A patient with a blood pressure of 155/88 mmHg is administered sublingual nitroglycerin for chest pain. Which physiological response is expected?

- A) A significant increase in blood pressure
- B) A decrease in blood pressure
- C) No change in blood pressure
- D) A decrease in respiratory rate

Answer: B) A decrease in blood pressure

Explanation:

Correct - B: NTG = venous + arterial vasodilator. ↓Preload + ↓Afterload = ↓BP. Also ↓myocardial O₂ demand. Check BP before each dose. Hold if SBP <90 or >30 drop.

Incorrect - A: Never increases BP. Contraindicated with Sildenafil - fatal hypotension.

Incorrect - C/D: BP always drops. RR may ↑ initially due to anxiety relief.

Trick: "Nitro = No pressure". Vasodilator = BP down. 3 doses 5min apart max.

Source: NORCET-2023 | Lippincott Pharma

Tags: #PYQ #HighYield #MedSurg #Pharma #Cardiac #NCLEX #SideEffect

[Med-Surg > Emergency > Shock] High - VERIFIED

Q47. In which type of shock is there a massive loss of sympathetic tone leading to decreased systemic vascular resistance and profound peripheral vasodilation?

- A) Cardiogenic shock
- B) Septic shock
- C) Hypovolemic shock
- D) Neurogenic shock

Answer: D) Neurogenic shock

Explanation:

Correct - D: Neurogenic shock = SCI T6 or above → loss of sympathetic outflow → unopposed parasympathetic → vasodilation + bradycardia + warm dry skin. Triad: Hypotension + Bradycardia + Warm skin.

Incorrect - A: Cardiogenic = pump failure → vasoconstriction + cold clammy.

Incorrect - B: Septic = early warm, late cold. Tachycardia present.

Incorrect - C: Hypovolemic = vasoconstriction + tachycardia + cold clammy.

Trick: "Neuro = No sympathetic = No squeeze = Warm + Slow HR". Only shock with bradycardia.

Source: NORCET-2023 | ATLS Guidelines

Tags: #PYQ #HighYield #MedSurg #Emergency #Shock #NCLEX #TrickQ

[Med-Surg > Pharma > Diabetes] Medium - VERIFIED

Q49. Which of the following is considered an intermediate-acting (basal) insulin used for maintenance therapy?

- A) Regular insulin
- B) NPH insulin
- C) Lispro
- D) Aspart

Answer: B) NPH insulin

Explanation:

Correct - B: NPH = Neutral Protamine Hagedorn. Onset 1-2h, Peak 4-12h, Duration 12-18h.

Cloudy. Basal insulin for between meals + overnight. Give BID.

Incorrect - A: Regular = Short acting. Onset 30min, Peak 2-4h, Duration 6-8h. Clear.

Incorrect - C/D: Lispro/Aspart = Rapid acting. Onset 15min, Peak 1h, Duration 3-5h. For meals.

Trick: "NPH = Not Particularly Hasty". Rapid = Lispro/Aspart/Glulisine. Long = Glargine/Detemir.

Source: NORCET-2023 | ADA Guidelines

Tags: #PYQ #HighYield #MedSurg #Pharma #Diabetes #Insulin #NCLEX

[Med-Surg > Pharma > Anticoagulants] High - VERIFIED

Q10. What is the correct pharmacological antidote for a Heparin overdose or toxicity?

- A) Warfarin
- B) Vitamin K
- C) Magnesium Sulfate
- D) Protamine Sulfate

Answer: D) Protamine Sulfate

Explanation:

Correct - D: Protamine sulfate 1mg neutralizes 100 units heparin. IV slow push. Check aPTT. Side effect: Hypotension, bradycardia.

Incorrect - A: Warfarin = another anticoagulant, not antidote.

Incorrect - B: Vit K = Warfarin antidote, not heparin.

Incorrect - C: MgSO₄ = Eclampsia, not anticoag reversal.

Trick: "Heparin-Hurt, Protamine-Protect". "Warfarin-Wrong, VitaminK-Korrect".

Source: NORCET AIIMS 2021 | Lippincott Pharma

Tags: #PYQ #HighYield #MedSurg #Pharma #Antidote #Emergency #NCLEX

[Med-Surg > Burns > Emergency] High - VERIFIED

Q14. A patient is brought to the emergency department with 40% total body surface area (TBSA) burns. What is the absolute priority nursing action?

- A) Prevent from infection
- B) Prevent from edema
- C) Assess for shock
- D) Administer IV fluid

Answer: D) Administer IV fluid

Explanation:

Correct - D: >20% TBSA = massive capillary leak → hypovolemic shock in 24-48hrs. Priority: 2 large bore IV + RL per Parkland. Assess shock karte raho par fluid START karo.

Incorrect - A/B: Infection/edema important but 24hr baad. First 24hr = FLUID.

Incorrect - C: Assess karte raho but ACTION = fluid dena.

Trick: "Burn >20% = Bolus first". ABCDE me C = Circulation = IV fluid.

Source: NORCET AIIMS 2021 | ATLS + Parkland

Tags: #PYQ #HighYield #MedSurg #Burns #Emergency #Priority #Shock

[Med-Surg > Cardiology > ECG] High - VERIFIED

Q21. Which specific electrocardiogram (ECG) change is recognized as a classic hallmark sign of severe hypokalemia?

- A) Tall, peaked T wave
- B) ST segment elevation
- C) Prolonged PR interval
- D) Prominent U wave

Answer: D) Prominent U wave

Explanation:

Correct - D: Hypokalemia ECG: Flat T wave + ST depression + Prominent U wave. U wave > T wave. PR prolonged, QRS wide. Risk: Torsades.

Incorrect - A: Peaked T = Hyperkalemia.

Incorrect - B: ST elevation = MI, not K⁺.

Trick: "Low K = Low T + U appears". "High K = High T". U for Under K⁺.

Source: NORCET AIIMS 2021 | Dale Dubin ECG

Tags: #PYQ #HighYield #MedSurg #Cardiology #ECG #Electrolytes #NCLEX

[Med-Surg > Immunization > COVID] Easy - VERIFIED

Q23. What specific type of vaccine is Covaxin, developed for protection against COVID-19?

- A) Live attenuated vaccine

- B) Inactivated vaccine
- C) Toxoid vaccine
- D) Recombinant vector vaccine

Answer: B) Inactivated vaccine

Explanation:

Correct - B: Covaxin = Whole virion inactivated vaccine by Bharat Biotech. Killed SARS-CoV-2 + Alhydroxyquim-II adjuvant. 2 doses 28 days apart.

Incorrect - A: No live COVID vaccine in India.

Incorrect - C: Toxoid = Tetanus, Diphtheria.

Incorrect - D: Covishield = ChAdOx1 vector vaccine.

Trick: "Covaxin = Killed Indian". "Covishield = Ship se aaya Vector".

Source: NORCET AIIMS 2021 | MoHFW COVID Guidelines

Tags: #PYQ #HighYield #MedSurg #Immunization #COVID #GK #Types

[Med-Surg > Emergency > IV Access] Medium - VERIFIED

Q24. A patient arrives in the emergency department suffering from massive blood loss after a severe road traffic accident. Which color intravenous (IV) cannula should the nurse immediately select for rapid fluid resuscitation?

- A) Green
- B) Grey
- C) Pink
- D) Blue

Answer: B) Grey

Explanation:

Correct - B: Grey = 16G, largest. Flow rate 180ml/min. Trauma/RTA/shock me 2 large bore 16G/14G needed. Poiseuille's law: Radius matters most.

Incorrect - A: Green = 18G, 90ml/min. Good but grey better for massive hemorrhage.

Incorrect - C: Pink = 20G, 60ml/min. Routine.

Incorrect - D: Blue = 22G, 36ml/min. Pediatrics only.

Trick: "Grey = Great for Gushing blood". "Green = General". Order: Orange14 > Grey16 > Green18 > Pink20 > Blue22.

Source: NORCET AIIMS 2021 | ATLS

Tags: #PYQ #HighYield #MedSurg #Emergency #IVAccess #Trauma #NCLEX

[Med-Surg > Pharma > ATT] Easy - VERIFIED

Q26. Which anti-tubercular medication is famous for causing the harmless side effect of turning the patient's urine orange-red?

- A) Ethambutol
- B) Rifampicin
- C) Isoniazid
- D) Streptomycin

Answer: B) Rifampicin

Explanation:

Correct - B: Rifampicin = Red urine, sweat, tears, saliva, contact lens. Harmless. Counsel patient warna panic. Also hepatotoxic.

Incorrect - A: Ethambutol = Eye toxicity, color vision loss.

Incorrect - C: INH = Peripheral neuropathy, hepatotoxicity. Give Vit B6.

Incorrect - D: Streptomycin = Ototoxicity, nephrotoxicity.

Trick: "Rifampicin = Red". "Ethambutol = Eye". "INH = Injury to Nerves".

Source: NORCET AIIMS 2021 | RNTCP Guidelines

Tags: #PYQ #HighYield #MedSurg #Pharma #TB #DOTS #SideEffect

[Med-Surg > Infectious > Tetanus] Easy - VERIFIED

Q29. Which of the following infectious diseases is classically and historically known as "Lock

Jaw"?

- A) Diphtheria
- B) Pertussis
- C) Tetanus
- D) Tuberculosis

Answer: C) Tetanus

Explanation:

Correct - C: Tetanus = Clostridium tetani toxin → trismus/lockjaw. First symptom: Masseter spasm. Then risus sardonicus, opisthotonus. Spores in soil/rust.

Incorrect - A: Diphtheria = Pseudomembrane throat.

Incorrect - B: Pertussis = Whooping cough.

Incorrect - D: TB = Chronic cough, hemoptysis.

Trick: "Tetanus = Tight jaw". Vaccine = TT in pregnancy, DTaP in kids.

Source: NORCET AIIMS 2021 | Park PSM

Tags: #PYQ #HighYield #MedSurg #Infectious #Tetanus #GK

[Med-Surg > Emergency > Shock] Medium - VERIFIED

Q2. Anaphylaxis is an example of which type of shock?

- A) Hypovolemic shock
- B) Distributive shock
- C) Obstructive shock
- D) Cardiogenic shock

Answer: B) Distributive shock

Explanation:

Correct - B: Anaphylaxis = Distributive shock. Massive vasodilation + ↑capillary permeability → fluid shifts to interstitial → ↓SVR + relative hypovolemia. Warm skin early.

Incorrect - A: Hypovolemic = blood/fluid loss. Cold, clammy.

Incorrect - C: Obstructive = PE, tamponade, tension pneumo. Physical block.

Incorrect - D: Cardiogenic = pump failure, MI. Cold, clammy.

Trick: "Distributive = Dilated vessels". Types: Septic, Anaphylactic, Neurogenic. All WARM early.

Source: NORCET-9 2025 | ATLS Guidelines

Tags: #PYQ #HighYield #MedSurg #Emergency #Shock #NCLEX #Conceptual

[Med-Surg > Neuro > Spinal Cord Injury] High - VERIFIED

Q3. When examining a patient who is paralyzed below the T4 level, what does the nurse expect to find?

- A) Flaccidity of the upper extremities
- B) Hyperflexion and spasticity of the upper extremities
- C) Impaired diaphragmatic function requiring ventilator support
- D) Independent use of the upper extremities and sufficient cough

Answer: D) Independent use of the upper extremities and sufficient cough

Explanation:

Correct - D: T4 level = below nipple line. Arms C5-T1 intact. Diaphragm C3-C5 intact. So upper limbs normal + cough intact + breathing ok. Below T4 paralyzed.

Incorrect - A/B: Upper limb issues only if cervical C1-C8 injury.

Incorrect - C: Diaphragm needs C3-C5. T4 injury = diaphragm works. C1-C3 = vent needed.

Trick: "T4 = Two Four = Thorax Free". Above T1 = arms gone. Below T6 = AD risk.

Source: NORCET-9 2025 | Lewis Med-Surg

Tags: #PYQ #HighYield #MedSurg #Neuro #SCI #Assessment #NCLEX

[Med-Surg > Neuro > Stroke] High - VERIFIED

Q10. What is the priority nursing assessment in the first 24 hours after the admission of a client with a thrombotic stroke?

- A) Cholesterol level

B) Pupil size and pupillary response

C) Bowel sounds

D) Echocardiogram

Answer: B) Pupil size and pupillary response

Explanation:

Correct - B: First 24-72hr = risk of ↑ICP, cerebral edema, herniation. Pupils = fastest indicator of neuro status + ICP. Unequal/fixed pupils = herniation emergency.

Incorrect - A: Cholesterol for long-term prevention, not 24hr priority.

Incorrect - C: Bowel sounds important but not life-saving in 24hr.

Incorrect - D: Echo for cardiac source, but neuro check first.

Trick: "Stroke = See pupils". PERRLA q1-2h first 24hr. Also LOC, GCS.

Source: NORCET-9 2025 | AHA Stroke Guidelines

Tags: #PYQ #HighYield #MedSurg #Neuro #Stroke #Priority #Emergency #NCLEX

[Med-Surg > Cardiology > Diagnostics] High - VERIFIED

Q19. Test to detect myocardial infarction:

A) ECG

B) Troponin T

C) CK-MB

D) Echocardiography

Answer: B) Troponin T

Explanation:

Correct - B: Troponin T/I = Gold standard, most specific. Rises 3-6hr, peaks 24hr, stays 7-14 days. >0.4ng/ml positive.

Incorrect - A: ECG shows changes but not specific. Can be normal in NSTEMI.

Incorrect - C: CK-MB rises 4-6hr, normalizes 48-72hr. Less specific.

Incorrect - D: Echo shows wall motion but not confirm MI.

Trick: "Troponin = True MI marker". First to rise Myoglobin, but Troponin most specific.

Source: NORCET-2020 | AHA MI Guidelines

Tags: #PYQ #HighYield #MedSurg #Cardiology #MI #Diagnostics #Lab

[Med-Surg > Emergency > Anaphylaxis] High - VERIFIED

Q24. Adrenaline dose in anaphylactic shock is:

A) 1:1000 (0.5 ml IM)

B) 1:10000 (5 ml IV)

C) 1:1000 (1 ml IV)

D) 1:10000 (10 ml IM)

Answer: A) 1:1000 (0.5 ml IM)

Explanation:

Correct - A: Anaphylaxis = 1:1000 Adrenaline 0.5ml IM anterolateral thigh. Repeat 5-15min. Adult. Pediatric 0.01mg/kg.

Incorrect - B: 1:10000 IV = cardiac arrest dose.

Incorrect - C: 1:1000 IV = death, cardiac arrest.

Incorrect - D: Wrong route and concentration.

Trick: "Anaphylaxis = IM 1:1000 in Thigh". "Cardiac arrest = IV 1:10000". IM for shock, IV for arrest.

Source: NORCET-2020 | AHA Anaphylaxis Guidelines

Tags: #PYQ #HighYield #MedSurg #Emergency #Anaphylaxis #Pharma #NCLEX

[Med-Surg > Respiratory > Pleural] Medium - VERIFIED

Q28. Condition with fluid and air in pleural cavity is:

A) Pneumothorax

B) Hydrothorax

C) Hydropneumothorax

D) Hemothorax

Answer: C) Hydropneumothorax

Explanation:

Correct - C: Hydropneumothorax = Fluid + Air in pleural space. X-ray: Air-fluid level. Causes: Trauma, TB, surgery.

Incorrect - A: Pneumothorax = Air only.

Incorrect - B: Hydrothorax/Pleural effusion = Fluid only.

Incorrect - D: Hemothorax = Blood only.

Trick: "Hydro = Water, Pneumo = Air". Both = Hydropneumothorax.

Source: NORCET-2020 | Respiratory Medicine

Tags: #PYQ #HighYield #MedSurg #Respiratory #Pleural #XRays

[Med-Surg > Stoma > Site Selection] Medium - VERIFIED

Q38. Correct statement about stoma formation:

A) Should be near bony prominence

B) Away from bony prominence

C) On scar tissue

D) Near umbilicus

Answer: B) Away from bony prominence

Explanation:

Correct - B: Stoma site = Away from bony prominences, scars, skin folds, waistline, umbilicus.

Flat area of rectus muscle, patient can see. Prevents leakage, skin breakdown.

Incorrect - A/C/D: Near bone/scar/umbilicus = pouch won't stick, leak hoga.

Trick: "Stoma = Smooth area pe". Bony area pe nahi lagta.

Source: NORCET-2020 | Stoma Care Guidelines

Tags: #PYQ #HighYield #MedSurg #Stoma #Surgical #Preop

[Med-Surg > Cardiology > ECG] High - VERIFIED

Q39. ECG showing ST depression and U wave indicates:

A) Hyperkalemia

B) Hypokalemia ($K^+ 2.2$ mEq/L)

C) Hyponatremia

D) Hypernatremia

Answer: B) Hypokalemia ($K^+ 2.2$ mEq/L)

Explanation:

Correct - B: Hypokalemia ECG: Flat T wave + ST depression + Prominent U wave + Prolonged QT.

$K^+ 2.2$ = severe hypokalemia. Risk: Torsades, cardiac arrest.

Incorrect - A: Hyperkalemia = Peaked T, QRS wide, sine wave.

Incorrect - C/D: Na^+ imbalance ECG changes nahi karta specific.

Trick: "Low K^+ = Low T + U wave". "High K^+ = High T". $K^+ 2.2$ = critical low.

Source: NORCET-2020 | Dale Dubin ECG

Tags: #PYQ #HighYield #MedSurg #Cardiology #ECG #Electrolytes #Emergency

[Med-Surg > Neuro > Seizure] High - VERIFIED

Q41. Immediate nursing action during an epileptic seizure is:

A) Insert airway immediately

B) Place patient in side-lying position

C) Restrain patient's movements

D) Give IV diazepam immediately

Answer: B) Place patient in side-lying position

Explanation:

Correct - B: Seizure priority: 1) Safety - side-lying to prevent aspiration + remove sharp objects.

2) Time it. 3) Loosen clothes. 4) O₂ after seizure. Don't insert anything in mouth.

Incorrect - A: Airway insertion during seizure = broken teeth, injury. Never force.

Incorrect - C: Restrain = fractures, injury. Guide movements only.
Incorrect - D: Diazepam if seizure >5min/status. Not immediate for single seizure.
Trick: "Seizure = Side me Sulao". Airway, IV, meds = after seizure stops.
Source: NORCET-2020 | Epilepsy Guidelines
Tags: #PYQ #HighYield #MedSurg #Neuro #Seizure #Emergency #Priority #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED

Q58. Which of the following is the antidote for organophosphorus poisoning?

- A) Naloxone
- B) Atropine
- C) Flumazenil
- D) Protamine sulfate

Answer: B) Atropine

Explanation:

Correct - B: OP poisoning = cholinergic crisis. Atropine = anticholinergic, dries secretions. Dose till atropinization: Clear lungs, HR >80, pupils dilate. Also PAM for nicotinic.

Incorrect - A: Naloxone = Opioid antidote.

Incorrect - C: Flumazenil = Benzodiazepine antidote.

Incorrect - D: Protamine = Heparin antidote.

Trick: "OP = Over Parasympathetic, Atropine = Anti Parasympathetic". Dry as bone, red as beet, mad as hatter.

Source: NORCET-2020 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #Antidote #OP #Emergency

[Med-Surg > Ophthalmology > Instruments] Easy - VERIFIED

Q59. Which of the following is used to measure intraocular pressure?

- A) Ophthalmoscope
- B) Tonometer
- C) Retinoscope
- D) Perimeter

Answer: B) Tonometer

Explanation:

Correct - B: Tonometer = measures IOP. Gold standard: Goldmann applanation. Normal IOP 10-21 mmHg. Glaucoma screening.

Incorrect - A: Ophthalmoscope = fundus exam, retina.

Incorrect - C: Retinoscope = refraction, power check.

Incorrect - D: Perimeter = visual field testing.

Trick: "Tonometer = Tension of eye". High IOP = Glaucoma.

Source: NORCET-2020 | Ophthalmology

Tags: #PYQ #HighYield #MedSurg #Ophthalmology #Instruments #Glaucoma

[Med-Surg > Toxicology > Antidotes] High - VERIFIED

Q61. Which of the following is the antidote for benzodiazepine overdose?

- A) Naloxone
- B) Flumazenil
- C) Atropine
- D) Protamine sulfate

Answer: B) Flumazenil

Explanation:

Correct - B: Flumazenil = competitive GABA-A antagonist. Reverses BZD sedation. IV 0.2mg over 30sec, repeat. Caution: Seizure risk if BZD dependent.

Incorrect - A: Naloxone = Opioid antidote.

Incorrect - C: Atropine = OP poisoning, bradycardia.

Incorrect - D: Protamine = Heparin antidote.

Trick: "Benzo = Benz Flumazenil". "Opioid = O Naloxone".

Source: NORCET-2020 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #Antidote #Emergency #NCLEX

[Med-Surg > Procedures > Bone Marrow] Medium - VERIFIED

Q62. Which of the following is the most common site for bone marrow aspiration in adults?

- A) Sternum
- B) Iliac crest
- C) Tibia
- D) Femur

Answer: B) Iliac crest

Explanation:

Correct - B: Posterior superior iliac crest = safest, most common. Flat bone, rich marrow, no vital organs. Anterior iliac crest also used.

Incorrect - A: Sternum = risk of cardiac tamponade, pericardial puncture. Avoided now.

Incorrect - C/D: Tibia/Femur = infants <2 years, not adults.

Trick: "Adult = Iliac, Infant = Tibia". Sternum dangerous.

Source: NORCET-2020 | Hematology Procedures

Tags: #PYQ #HighYield #MedSurg #Procedures #BoneMarrow #Safety

[Med-Surg > GI > Peptic Ulcer] High - VERIFIED

Q65. Which of the following is the most common cause of peptic ulcer?

- A) Stress
- B) H. pylori infection
- C) Smoking
- D) Alcohol

Answer: B) H. pylori infection

Explanation:

Correct - B: H. pylori = 90% duodenal ulcers, 70% gastric ulcers. Others: NSAIDs 2nd common.

Treat with Triple/Quadruple therapy.

Incorrect - A: Stress = aggravates, not primary cause. Old myth.

Incorrect - C/D: Smoking/Alcohol = risk factors, not primary cause.

Trick: "Ulcer = H. pylori Unless NSAID history". Test: Urea breath, stool antigen.

Source: NORCET-2020 | Harrison Medicine

Tags: #PYQ #HighYield #MedSurg #GI #PepticUlcer #HPylori #NCLEX

[Med-Surg > Cardiology > MI] High - VERIFIED

Q69. Which of the following is the most common cause of death in myocardial infarction?

- A) Arrhythmia
- B) Heart failure
- C) Cardiogenic shock
- D) Cardiac rupture

Answer: A) Arrhythmia

Explanation:

Correct - A: Arrhythmia = #1 cause of death in MI, esp first 24hrs. VF/VT most lethal. 50% die before hospital due to VF.

Incorrect - B: Heart failure = late cause.

Incorrect - C: Cardiogenic shock = 5-8% mortality.

Incorrect - D: Rupture = 3-5 days post MI, less common.

Trick: "MI me First killer = VF". That's why AED/defib important.

Source: NORCET-2020 | AHA MI Guidelines

Tags: #PYQ #HighYield #MedSurg #Cardiology #MI #Emergency #Death

[Med-Surg > Burns > Complications] High - VERIFIED

Q70. Which of the following is the most common cause of death in burns?

- A) Shock
- B) Sepsis
- C) Fluid loss
- D) Pain

Answer: B) Sepsis

Explanation:

Correct - B: Sepsis/Infection = #1 cause of death in burns >48hrs. Burn wound = open, immune suppressed. Pseudomonas, Staph aureus.

Incorrect - A: Shock = immediate death <48hrs in >40% TBSA.

Incorrect - C: Fluid loss leads to shock, but sepsis kills later.

Incorrect - D: Pain not fatal.

Trick: "Burn Early = Shock, Burn Late = Sepsis". First 48hr shock, after 48hr sepsis.

Source: NORCET-2020 | Burn Management

Tags: #PYQ #HighYield #MedSurg #Burns #Sepsis #Mortality #NCLEX

[Med-Surg > Endocrine > DM Complications] High - VERIFIED

Q71. Which of the following is the most common cause of death in diabetes mellitus?

- A) Hypoglycemia
- B) Diabetic ketoacidosis
- C) Myocardial infarction
- D) Stroke

Answer: C) Myocardial infarction

Explanation:

Correct - C: DM = CAD risk 2-4x. MI = #1 cause of death in DM, 65-80%. Macrovascular complication.

Incorrect - A: Hypoglycemia can kill but less common than MI.

Incorrect - B: DKA = Type 1 main, but MI overall more common.

Incorrect - D: Stroke 2nd common.

Trick: "Diabetes = Dil ka Dushman". MI > Stroke > CKD.

Source: NORCET-2020 | ADA Guidelines

Tags: #PYQ #HighYield #MedSurg #Endocrine #DM #Mortality #Cardiac

[Med-Surg > Cardiology > Hypertension] High - VERIFIED

Q72. Which of the following is the most common cause of death in hypertension?

- A) Stroke
- B) Myocardial infarction
- C) Heart failure
- D) Renal failure

Answer: A) Stroke

Explanation:

Correct - A: HTN = #1 risk factor for Stroke/CVA. Hemorrhagic + Ischemic. Stroke > MI in HTN worldwide.

Incorrect - B: MI 2nd common.

Incorrect - C/D: HF/CKD later complications.

Trick: "HTN = Head ka Tension = Stroke". BP control = stroke prevention.

Source: NORCET-2020 | JNC 8 Guidelines

Tags: #PYQ #HighYield #MedSurg #Cardiology #HTN #Stroke #Mortality

[Med-Surg > Oncology > Mortality] Medium - VERIFIED

Q73. Which of the following is the most common cause of death in cancer patients?

- A) Cachexia
- B) Infection

C) Metastasis

D) Organ failure

Answer: C) Metastasis

Explanation:

Correct - C: Metastasis = 90% cancer deaths. Spread to vital organs - liver, lung, brain, bone.

Organ failure due to mets.

Incorrect - A: Cachexia contributes but metastasis main.

Incorrect - B: Infection common in chemo but mets #1.

Incorrect - D: Organ failure due to metastasis.

Trick: "Cancer Kills by Spreading = Metastasis". Local tumor rarely fatal.

Source: NORCET-2020 | Oncology

Tags: #PYQ #HighYield #MedSurg #Oncology #Metastasis #Mortality

[Med-Surg > Renal > ARF/CKD] High - VERIFIED

Q74. Which of the following is the most common cause of death in renal failure?

A) Hyperkalemia

B) Acidosis

C) Fluid overload

D) Hypertension

Answer: A) Hyperkalemia

Explanation:

Correct - A: Renal failure = can't excrete K⁺ → Hyperkalemia → VF/Asystole → sudden cardiac death. K⁺ >6.5 lethal.

Incorrect - B/C/D: Acidosis, fluid overload, HTN contribute but HyperK kills fastest.

Trick: "Renal Failure = K⁺ Failure = Heart Failure". Check K⁺ daily in ARF.

Source: NORCET-2020 | Nephrology

Tags: #PYQ #HighYield #MedSurg #Renal #ARF #Hyperkalemia #Emergency #NCLEX

[Med-Surg > GI > Cirrhosis] High - VERIFIED

Q75. Which of the following is the most common cause of death in cirrhosis?

A) Hepatic encephalopathy

B) Variceal bleeding

C) Ascites

D) Infection

Answer: B) Variceal bleeding

Explanation:

Correct - B: Cirrhosis → Portal HTN → Esophageal varices → Massive UGIB → Hypovolemic shock death. #1 cause.

Incorrect - A: HE 2nd common but variceal bleed kills faster.

Incorrect - C: Ascites = SBP risk but bleed #1.

Incorrect - D: Infection SBP common but bleed more fatal.

Trick: "Cirrhosis = Choke on Blood". Variceal bleed = emergency.

Source: NORCET-2020 | Hepatology

Tags: #PYQ #HighYield #MedSurg #GI #Cirrhosis #Varices #Mortality

[Med-Surg > Respiratory > COPD] High - VERIFIED

Q76. Which of the following is the most common cause of death in COPD?

A) Respiratory failure

B) Pneumonia

C) Pulmonary embolism

D) Heart failure

Answer: A) Respiratory failure

Explanation:

Correct - A: COPD = progressive airflow limitation → Type 2 resp failure → CO₂ retention →

death. Acute exacerbation with hypercapnia.
Incorrect - B: Pneumonia precipitates but resp failure kills.
Incorrect - C/D: PE/Cor pulmonale less common than resp failure.
Trick: "COPD = Can't Oxygenate Properly, Die". End stage = resp failure.
Source: NORCET-2020 | GOLD Guidelines
Tags: #PYQ #HighYield #MedSurg #Respiratory #COPD #Mortality

[Med-Surg > Respiratory > Asthma] High - VERIFIED

Q77. Which of the following is the most common cause of death in asthma?

- A) Status asthmaticus
- B) Pneumonia
- C) Respiratory failure
- D) Pulmonary embolism

Answer: A) Status asthmaticus

Explanation:

Correct - A: Status asthmaticus = severe asthma unresponsive to treatment >24hrs. Leads to resp failure, pneumothorax, death. Silent chest ominous.

Incorrect - B/C: Pneumonia/resp failure due to status, but status is primary.

Trick: "Asthma death = Status attack". Silent chest = danger.

Source: NORCET-2020 | GINA Guidelines

Tags: #PYQ #HighYield #MedSurg #Respiratory #Asthma #Emergency #Mortality

[Med-Surg > Infectious > HIV] High - VERIFIED

Q78. Which of the following is the most common cause of death in HIV/AIDS?

- A) Opportunistic infections
- B) Malignancy
- C) Wasting syndrome
- D) Neurological complications

Answer: A) Opportunistic infections

Explanation:

Correct - A: AIDS = CD4 <200 → OI: PCP pneumonia, TB, Toxoplasmosis, Cryptococcal meningitis, CMV. OI = #1 cause of death.

Incorrect - B: Malignancy = Kaposi, Lymphoma 2nd common.

Incorrect - C/D: Wasting, neuro less common than OI.

Trick: "AIDS = All Infections Dangerous". PCP pneumonia #1 OI.

Source: NORCET-2020 | NACO Guidelines

Tags: #PYQ #HighYield #MedSurg #Infectious #HIV #OI #Mortality

[Med-Surg > Infectious > TB] High - VERIFIED

Q79. Which of the following is the most common cause of death in tuberculosis?

- A) Hemoptysis
- B) Respiratory failure
- C) Disseminated disease
- D) Drug resistance

Answer: C) Disseminated disease

Explanation:

Correct - C: Miliary/Disseminated TB = spread to liver, spleen, bone marrow, meninges → multi-organ failure death. Esp in HIV.

Incorrect - A: Hemoptysis scary but rarely fatal.

Incorrect - B: Resp failure in extensive pulmonary but disseminated more fatal.

Incorrect - D: MDR/XDR TB cause but dissemination mechanism of death.

Trick: "TB death = Spread everywhere = Disseminated". Miliary = millet seeds on Xray.

Source: NORCET-2020 | RNTCP Guidelines

Tags: #PYQ #HighYield #MedSurg #Infectious #TB #Mortality #Miliary

[Med-Surg > Infectious > Malaria] High - VERIFIED

Q80. Which of the following is the most common cause of death in malaria?

- A) Cerebral malaria
- B) Anemia
- C) Hypoglycemia
- D) Renal failure

Answer: A) Cerebral malaria

Explanation:

Correct - A: *P. falciparum* → Cerebral malaria = seizures, coma, death. RBC sequestration in brain vessels. #1 fatal complication.

Incorrect - B/C/D: Anemia, hypoglycemia, ARF complications but cerebral malaria kills fastest.

Trick: "Falciparum = Fatal to Brain". Cerebral malaria = death.

Source: NORCET-2020 | NVBDCP Guidelines

Tags: #PYQ #HighYield #MedSurg #Infectious #Malaria #Falciparum #Mortality

[Med-Surg > Infectious > Dengue] High - VERIFIED

Q81. Which of the following is the most common cause of death in dengue fever?

- A) Shock
- B) Hemorrhage
- C) Encephalitis
- D) Renal failure

Answer: A) Shock

Explanation:

Correct - A: Dengue = Dengue Shock Syndrome DSS. Plasma leakage → hypovolemic shock → death. Hemorrhage 2nd common in DHF. Warning signs: Abdominal pain, vomiting, mucosal bleed, lethargy.

Incorrect - B: Hemorrhage in DHF but shock #1 killer.

Incorrect - C/D: Encephalitis/ARF rare causes.

Trick: "Dengue Death = Shock". DSS = Shock, DHF = Bleed. Shock kills faster.

Source: NORCET-2020 | NVBDCP Dengue Guidelines

Tags: #PYQ #HighYield #MedSurg #Infectious #Dengue #Mortality #Shock #NCLEX

[Med-Surg > Infectious > Typhoid] High - VERIFIED

Q82. Which of the following is the most common cause of death in typhoid fever?

- A) Intestinal perforation
- B) Hemorrhage
- C) Septicemia
- D) Pneumonia

Answer: A) Intestinal perforation

Explanation:

Correct - A: Typhoid = *Salmonella typhi* → Peyer's patches necrosis in ileum → Perforation in 3rd week → Peritonitis → Death. #1 fatal complication.

Incorrect - B: Hemorrhage 2nd common, from ulcerated vessels.

Incorrect - C/D: Septicemia/Pneumonia less common than perforation.

Trick: "Typhoid = Tear in Intestine". 3rd week danger. Guarding, rigidity, absent bowel sounds.

Source: NORCET-2020 | Park PSM

Tags: #PYQ #HighYield #MedSurg #Infectious #Typhoid #Perforation #Mortality

[Med-Surg > Infectious > Diphtheria] High - VERIFIED

Q84. Which of the following is the most common cause of death in diphtheria?

- A) Myocarditis
- B) Respiratory obstruction

C) Septicemia

D) Renal failure

Answer: B) Respiratory obstruction

Explanation:

Correct - B: Diphtheria = thick gray pseudomembrane in throat → Airway obstruction → Asphyxia death. Emergency tracheostomy needed. Bull neck sign.

Incorrect - A: Myocarditis 2nd common, 2-3 weeks later due to toxin.

Incorrect - C/D: Septicemia/Renal failure rare.

Trick: "Diphtheria = Death by blocked airway". Membrane kills before myocarditis.

Source: NORCET-2020 | Park PSM

Tags: #PYQ #HighYield #MedSurg #Infectious #Diphtheria #Airway #Emergency

[Med-Surg > Infectious > Tetanus] High - VERIFIED

Q86. Which of the following is the most common cause of death in tetanus?

A) Respiratory failure

B) Cardiac arrest

C) Sepsis

D) Renal failure

Answer: A) Respiratory failure

Explanation:

Correct - A: Tetanus = Generalized spasms → Laryngospasm + Resp muscle spasm + Aspiration → Resp failure. #1 cause. Also autonomic dysfunction.

Incorrect - B: Cardiac arrest due to autonomic storm 2nd common.

Incorrect - C/D: Sepsis/Renal rare.

Trick: "Tetanus = Tight muscles = Can't breathe". Laryngospasm kills.

Source: NORCET-2020 | Park PSM

Tags: #PYQ #HighYield #MedSurg #Infectious #Tetanus #RespFailure #Mortality

[Med-Surg > Infectious > Rabies] High - VERIFIED

Q87. Which of the following is the most common cause of death in rabies?

A) Respiratory failure

B) Cardiac arrest

C) Sepsis

D) Renal failure

Answer: A) Respiratory failure

Explanation:

Correct - A: Rabies = Encephalitis → Resp muscle paralysis + Aspiration + Brainstem involvement → Resp failure. 100% fatal once symptoms. Hydrophobia, aerophobia.

Incorrect - B: Cardiac arrest terminal event but resp failure primary.

Incorrect - C/D: Sepsis/Renal not main cause.

Trick: "Rabies = Resp arrest". Brain stem death.

Source: NORCET-2020 | NVBDCP Rabies Guidelines

Tags: #PYQ #HighYield #MedSurg #Infectious #Rabies #Mortality #NCLEX

[Med-Surg > Emergency > Snake Bite] High - VERIFIED

Q89. Which of the following is the most common cause of death in snake bite?

A) Respiratory failure

B) Cardiac arrest

C) Renal failure

D) Sepsis

Answer: A) Respiratory failure

Explanation:

Correct - A: Neurotoxic snake (Cobra, Krait) = Neuromuscular paralysis → Resp failure. #1 cause. Also ASV anaphylaxis. Viper = Bleeding + ARF.

Incorrect - B: Cardiac arrest secondary to resp failure/hyperK.

Incorrect - C: Renal failure in viper, 2nd common.

Incorrect - D: Sepsis late.

Trick: "Neurotoxic = No breathing". Cobra/Krait = Resp failure. Viper = Bleed + Kidney.

Source: NORCET-2020 | Snake Bite Management

Tags: #PYQ #HighYield #MedSurg #Emergency #SnakeBite #RespFailure #Mortality

[Med-Surg > Emergency > Scorpion Sting] High - VERIFIED

Q90. Which of the following is the most common cause of death in scorpion sting?

A) Myocarditis

B) Respiratory failure

C) Shock

D) Sepsis

Answer: C) Shock

Explanation:

Correct - C: Scorpion venom = Autonomic storm → Massive catecholamine release → Cardiogenic shock + Pulmonary edema → Death. Prazosin drug of choice.

Incorrect - A: Myocarditis occurs but shock kills.

Incorrect - B: Resp failure secondary to pulmonary edema.

Incorrect - D: Sepsis not main.

Trick: "Scorpion = Shock + Prazosin". Autonomic storm → cardiogenic shock.

Source: NORCET-2020 | Emergency Toxicology

Tags: #PYQ #HighYield #MedSurg #Emergency #ScorpionSting #Shock #Mortality

[Med-Surg > Emergency > Bee Sting] High - VERIFIED

Q91. Which of the following is the most common cause of death in bee sting?

A) Anaphylaxis

B) Shock

C) Respiratory failure

D) Cardiac arrest

Answer: A) Anaphylaxis

Explanation:

Correct - A: Bee venom = Type 1 hypersensitivity in sensitized → Anaphylaxis → Airway edema + Bronchospasm + Shock → Death in minutes. Adrenaline IM immediate.

Incorrect - B/C/D: Shock/Resp failure/Cardiac arrest due to anaphylaxis, but anaphylaxis primary.

Trick: "Bee = Be Anaphylactic". Single sting can kill in allergic.

Source: NORCET-2020 | Anaphylaxis Guidelines

Tags: #PYQ #HighYield #MedSurg #Emergency #BeeSting #Anaphylaxis #Mortality

[Med-Surg > Toxicology > Overdose] High - VERIFIED

Q92. Which of the following is the most common cause of death in drug overdose?

A) Respiratory depression

B) Cardiac arrest

C) Sepsis

D) Renal failure

Answer: A) Respiratory depression

Explanation:

Correct - A: Opioids, BZD, Alcohol, Barbiturates = CNS depressants → Resp depression → Hypoxia → Death. #1 mechanism. Naloxone for opioid.

Incorrect - B: Cardiac arrest secondary to resp depression/hypoxia.

Incorrect - C/D: Sepsis/Renal late/rare.

Trick: "Overdose = O2 Down = Resp depression". Pinpoint pupils + coma + resp depression = Opioid triad.

Source: NORCET-2020 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #Overdose #RespDepression #Mortality

[Med-Surg > Surgical > Complications] High - VERIFIED

Q94. Which of the following is the most common cause of death in surgery?

- A) Hemorrhage
- B) Sepsis
- C) Shock
- D) Cardiac arrest

Answer: B) Sepsis

Explanation:

Correct - B: Post-op sepsis = #1 cause of death after 48hrs. SSI, anastomotic leak, pneumonia, UTI → MODS. Early death = hemorrhage/shock.

Incorrect - A: Hemorrhage = immediate intra-op/24hr death.

Incorrect - C: Shock due to hemorrhage/sepsis.

Incorrect - D: Cardiac arrest due to hypoxia/bleeding.

Trick: "Surgery Early = Bleed, Surgery Late = Sepsis". Sepsis overall #1.

Source: NORCET-2020 | Surgical Complications

Tags: #PYQ #HighYield #MedSurg #Surgical #Sepsis #Mortality #Postop

[Med-Surg > Trauma > Mortality] High - VERIFIED

Q95. Which of the following is the most common cause of death in trauma?

- A) Hemorrhage
- B) Shock
- C) Sepsis
- D) Respiratory failure

Answer: A) Hemorrhage

Explanation:

Correct - A: Trauma = Exsanguination/hemorrhage #1 cause. First 1hr "Golden hour". Preventable death. Head injury 2nd.

Incorrect - B: Shock due to hemorrhage, but hemorrhage primary.

Incorrect - C: Sepsis late >48hrs.

Incorrect - D: Resp failure due to chest trauma/ARDS.

Trick: "Trauma = Bleeding to Death". Golden hour, stop bleed, TXA, blood.

Source: NORCET-2020 | ATLS Guidelines

Tags: #PYQ #HighYield #MedSurg #Trauma #Hemorrhage #Mortality #Emergency

[Med-Surg > Emergency > RTA] High - VERIFIED

Q96. Which of the following is the most common cause of death in road traffic accidents?

- A) Head injury
- B) Hemorrhage
- C) Shock
- D) Sepsis

Answer: A) Head injury

Explanation:

Correct - A: RTA = TBI/Head injury #1 cause of death. Diffuse axonal injury, SDH, EDH. Unhelmeted 2-wheeler.

Incorrect - B: Hemorrhage 2nd common, abdominal/thoracic/pelvic.

Incorrect - C: Shock due to hemorrhage/head injury.

Incorrect - D: Sepsis late.

Trick: "RTA = Road pe Head phata". Head injury > Hemorrhage in RTA specifically.

Source: NORCET-2020 | Trauma Registry

Tags: #PYQ #HighYield #MedSurg #Emergency #RTA #TBI #Mortality

[Med-Surg > Emergency > Drowning] High - VERIFIED

Q97. Which of the following is the most common cause of death in drowning?

- A) Respiratory failure
- B) Cardiac arrest
- C) Shock
- D) Sepsis

Answer: A) Respiratory failure

Explanation:

Correct - A: Drowning = Aspiration → Hypoxia → Resp failure/ARDS → Death. Wet drowning 85%, Dry drowning laryngospasm 15%.

Incorrect - B: Cardiac arrest secondary to hypoxia.

Incorrect - C/D: Shock/Sepsis late.

Trick: "Drowning = Drowned lungs = Resp failure". Hypoxia kills.

Source: NORCET-2020 | Emergency Medicine

Tags: #PYQ #HighYield #MedSurg #Emergency #Drowning #RespFailure #Mortality

[Med-Surg > Emergency > Electrocution] High - VERIFIED

Q98. Which of the following is the most common cause of death in electrocution?

- A) Cardiac arrest
- B) Respiratory failure
- C) Shock
- D) Sepsis

Answer: A) Cardiac arrest

Explanation:

Correct - A: Electrocution = VF/Asystole due to current across heart. #1 cause immediate death.

Also resp arrest if brainstem hit.

Incorrect - B: Resp failure if brainstem/medulla hit, 2nd common.

Incorrect - C/D: Shock/Sepsis not immediate.

Trick: "Electric = Heart Attack". Current → VF → Death. CPR immediate.

Source: NORCET-2020 | Emergency Medicine

Tags: #PYQ #HighYield #MedSurg #Emergency #Electrocution #CardiacArrest #Mortality

[Med-Surg > Oncology > Radiation] Medium - VERIFIED

Q99. Which of the following is the most common cause of death in radiation exposure?

- A) Bone marrow suppression
- B) Sepsis
- C) Shock
- D) Respiratory failure

Answer: A) Bone marrow suppression

Explanation:

Correct - A: Acute Radiation Syndrome = Bone marrow syndrome at 2-8 Gy. Pancytopenia → Infection + Bleeding → Death. GI syndrome >8 Gy. CNS >30 Gy.

Incorrect - B: Sepsis due to bone marrow suppression, but suppression primary.

Incorrect - C/D: Shock/Resp failure not primary.

Trick: "Radiation = Ruins Bone marrow". Pancytopenia kills.

Source: NORCET-2020 | Radiation Emergency

Tags: #PYQ #HighYield #MedSurg #Oncology #Radiation #BoneMarrow #Mortality

[Med-Surg > Pharma > Anticoagulants] High - VERIFIED

Q1. A nurse is assessing a patient who has just been started on heparin therapy. Which of the following laboratory tests should be monitored regularly?

- A) International Normalized Ratio (INR)
- B) Prothrombin time (PT)
- C) Partial thromboplastin time (PTT)

D) Platelet count

Answer: C) Partial thromboplastin time (PTT)

Explanation:

Correct - C: Heparin/UFH = monitor aPTT/PTT. Therapeutic range 1.5-2.5x control (60-80 sec).

Also check platelets for HIT.

Incorrect - A/B: INR/PT = Warfarin monitoring, not heparin.

Incorrect - D: Platelet count for HIT but PTT is primary.

Trick: "Heparin = aPTT, Warfarin = INR/PT". LMWH no routine monitoring.

Source: NORCET NOV 2021 | Anticoagulation Guidelines

Tags: #PYQ #HighYield #MedSurg #Pharma #Heparin #Lab #Monitoring #NCLEX

[Med-Surg > Pharma > Steroids] High - VERIFIED

Q3. A nurse is caring for a patient who is receiving a high dose of corticosteroids. Which of the following is a common side effect of corticosteroid therapy?

A) Weight loss

B) Muscle weakness

C) Hypertension

D) Hypoglycemia

Answer: C) Hypertension

Explanation:

Correct - C: Steroids = Na⁺ & water retention → Hypertension, Cushingoid features. Also hyperglycemia, osteoporosis, peptic ulcer, immunosuppression, weight gain.

Incorrect - A: Weight GAIN, not loss.

Incorrect - B: Muscle weakness with chronic use, but HTN more common acute.

Incorrect - D: HYPERglycemia, not hypo.

Trick: "Steroids = Swell, Sugar high, BP high". Cushingoid = moon face, buffalo hump.

Source: NORCET NOV 2021 | Pharma Side Effects

Tags: #PYQ #HighYield #MedSurg #Pharma #Steroids #SideEffects #NCLEX

[Med-Surg > Cardiology > HTN] Easy - VERIFIED

Q4. The nurse is providing education to a patient diagnosed with hypertension. Which of the following should the nurse recommend as a lifestyle change?

A) Increasing sodium intake

B) Reducing physical activity

C) Eating a low-fat, low-sodium diet

D) Decreasing fluid intake

Answer: C) Eating a low-fat, low-sodium diet

Explanation:

Correct - C: HTN lifestyle: DASH diet - low Na <2.3g/day, low fat, ↑fruits/veg, ↑exercise, ↓alcohol, quit smoking, weight loss.

Incorrect - A: ↑Sodium = ↑BP.

Incorrect - B: ↓Activity = ↑BP. Need exercise.

Incorrect - D: ↓Fluid not needed unless HF.

Trick: "DASH Diet = Dietary Approaches to Stop Hypertension". Low salt, high potassium.

Source: NORCET-2021 | JNC 8 Guidelines

Tags: #PYQ #HighYield #MedSurg #Cardiology #HTN #PatientEducation #DASH

[Med-Surg > Cardiology > HF] High - VERIFIED

Q5. A patient with chronic heart failure is prescribed a diuretic. The nurse should closely monitor the patient for signs of:

A) Hyperkalemia

B) Hypokalemia

C) Hypoglycemia

D) Hypernatremia

Answer: B) Hypokalemia

Explanation:

Correct - B: Loop diuretics Furosemide, Thiazides = K⁺ wasting → Hypokalemia. S/S: Muscle weakness, U waves, arrhythmia, digoxin toxicity risk. Monitor K⁺, give K⁺ supplements/K-sparing.

Incorrect - A: Hyperkalemia with K-sparing diuretics spironolactone.

Incorrect - C/D: Hypoglycemia/Hypernatremia not primary.

Trick: "Loop diuretics = Lose K⁺". "K-sparing = Keep K⁺". HF + Digoxin + Furosemide = watch K⁺.

Source: NORCET-2021 | Heart Failure Management

Tags: #PYQ #HighYield #MedSurg #Cardiology #HF #Diuretics #Electrolytes #NCLEX

[Med-Surg > Neuro > Stroke] Medium - VERIFIED

Q7. A nurse is caring for a patient after a stroke. What is the primary goal of rehabilitation for this patient?

A) To restore the patient to pre-stroke condition

B) To prevent further strokes

C) To assist the patient in regaining as much functional ability as possible

D) To improve the patient's cognitive abilities

Answer: C) To assist the patient in regaining as much functional ability as possible

Explanation:

Correct - C: Stroke rehab goal = Maximize independence, ADL, quality of life. Full restoration often not possible. Realistic: adapt, compensate, prevent complications.

Incorrect - A: Pre-stroke restoration unrealistic for most.

Incorrect - B: Prevention secondary, not rehab primary goal.

Incorrect - D: Cognitive only if affected, not primary for all.

Trick: "Rehab = Regain function, not Restore old". ADL independence key.

Source: NORCET-2021 | Stroke Rehab Guidelines

Tags: #PYQ #HighYield #MedSurg #Neuro #Stroke #Rehab #Goal

[Med-Surg > Respiratory > Asthma] Medium - VERIFIED

Q8. A nurse is educating a patient with asthma on how to use a metered-dose inhaler (MDI).

Which of the following actions should the nurse include in the teaching?

A) Shake the inhaler vigorously before using it

B) Inhale deeply and hold your breath for 10 seconds

C) Use the inhaler only during an asthma attack

D) Exhale forcefully before using the inhaler

Answer: A) Shake the inhaler vigorously before using it

Explanation:

Correct - A: MDI steps: 1) Shake well 5sec 2) Exhale 3) Seal lips 4) Press + Inhale slow deep 5) Hold breath 10sec 6) Wait 1min between puffs. Shake mixes drug.

Incorrect - B: Hold breath correct but A is first step mentioned.

Incorrect - C: Use as controller + rescue, not only attack.

Incorrect - D: Exhale gently, not forcefully.

Trick: "Shake, Exhale, Inhale, Hold". Shake first for suspension MDIs.

Source: NORCET-2021 | Asthma Education

Tags: #PYQ #HighYield #MedSurg #Respiratory #Asthma #MDI #PatientEducation

[Med-Surg > Respiratory > PE] High - VERIFIED

Q9. A nurse is assessing a patient with a suspected pulmonary embolism (PE). Which of the following is the most common presenting symptom of a PE?

A) Chest pain

B) Tachypnea

C) Hemoptysis

D) Fever

Answer: B) Tachypnea

Explanation:

Correct - B: PE = #1 symptom: Tachypnea >90% cases. Then dyspnea, chest pain pleuritic, tachycardia, anxiety. Hemoptysis only 30%.

Incorrect - A: Chest pain 2nd common.

Incorrect - C: Hemoptysis <30%.

Incorrect - D: Fever low grade, not primary.

Trick: "PE = Panic + Panting". Tachypnea most sensitive sign. Sudden onset SOB.

Source: NORCET-2021 | PE Guidelines

Tags: #PYQ #HighYield #MedSurg #Respiratory #PE #Emergency #NCLEX

[Med-Surg > Surgical > Mastectomy] High - VERIFIED

Q10. A nurse is caring for a patient who is post-operative following a mastectomy. Which of the following is a priority intervention?

A) Encourage the patient to cough and deep breathe every hour

B) Assess the surgical site for signs of infection every shift

C) Provide a pillow for the patient to place under the arm for support

D) Educate the patient about exercises for the arm and shoulder

Answer: A) Encourage the patient to cough and deep breathe every hour

Explanation:

Correct - A: Post-op priority = Prevent atelectasis/pneumonia. GA + pain → shallow breathing.

Cough deep breathe q1h, incentive spirometer. ABC priority.

Incorrect - B: Assess site important but ABC first. Infection later.

Incorrect - C: Pillow/elevation for edema, but breathing priority.

Incorrect - D: Exercises day 1-2, but breathing immediate priority.

Trick: "Post-op = Pulmonary first". Cough, deep breathe, turn, leg exercises. Airway before wound.

Source: NORCET-2021 | Postop Care

Tags: #PYQ #HighYield #MedSurg #Surgical #Mastectomy #Postop #Priority #NCLEX

[Med-Surg > Respiratory > COPD] High - VERIFIED

Q12. A patient with COPD is receiving oxygen therapy. Which of the following oxygen delivery devices is most appropriate?

A) Nasal cannula

B) Non-rebreather mask

C) Venturi mask

D) Simple face mask

Answer: C) Venturi mask

Explanation:

Correct - C: COPD = Chronic CO₂ retainers, hypoxic drive. Need precise low FiO₂ 24-28%. Venturi mask = exact FiO₂, prevents CO₂ narcosis. Target SpO₂ 88-92%.

Incorrect - A: Nasal cannula 1-6L = 24-44% uncontrolled, risk CO₂ retention if high flow.

Incorrect - B: NRBM = 90-100% too high, knocks out hypoxic drive.

Incorrect - D: Simple mask 40-60% uncontrolled.

Trick: "COPD = Controlled O₂ = Venturi". Never give >2L NC or high FiO₂ without ABG.

Source: NORCET-2021 | GOLD Guidelines

Tags: #PYQ #HighYield #MedSurg #Respiratory #COPD #OxygenTherapy #Venturi #NCLEX

[Med-Surg > Endocrine > DM] High - VERIFIED

Q13. A nurse is caring for a patient with diabetes mellitus. Which of the following is a sign of hypoglycemia?

A) Polyuria

B) Polydipsia

- C) Sweating
- D) Weight gain

Answer: C) Sweating

Explanation:

Correct - C: Hypoglycemia <70mg/dL: Autonomic S/S = Sweating, tremors, tachycardia, hunger, anxiety. Neuroglycopenic = confusion, dizziness, seizure, coma.

Incorrect - A/B: Polyuria, Polydipsia = HYPERglycemia.

Incorrect - D: Weight gain = insulin/hyperglycemia long term.

Trick: "Hypo = Hunger, Hypo, Sweating, Shaky". "Hyper = 3 P: Polyuria, Polydipsia, Polyphagia".

Source: NORCET-2021 | ADA Guidelines

Tags: #PYQ #HighYield #MedSurg #Endocrine #DM #Hypoglycemia #NCLEX

[Med-Surg > Renal > Anemia] High - VERIFIED

Q14. A patient with chronic renal failure is prescribed erythropoietin. The nurse should monitor for which complication?

- A) Hypertension
- B) Hypotension
- C) Hypoglycemia
- D) Hyperkalemia

Answer: A) Hypertension

Explanation:

Correct - A: EPO/Erythropoietin for renal anemia → ↑Hct → ↑blood viscosity → Hypertension, seizures, thrombotic events. Target Hb 10-11.5, not >12. Monitor BP, Hct.

Incorrect - B: Hypotension not EPO side effect.

Incorrect - C/D: Hypoglycemia/Hyperkalemia not EPO related.

Trick: "EPO = Elevated Pressure Often". Also clots, stroke risk if Hct >12.

Source: NORCET-2021 | CKD Guidelines

Tags: #PYQ #HighYield #MedSurg #Renal #CKD #EPO #HTN #NCLEX

[Med-Surg > GI > Cirrhosis] High - VERIFIED

Q15. A nurse is caring for a patient with cirrhosis. Which of the following findings indicates hepatic encephalopathy?

- A) Ascites
- B) Asterixis
- C) Jaundice
- D) Variceal bleeding

Answer: B) Asterixis

Explanation:

Correct - B: Hepatic encephalopathy = ↑ammonia → neuro changes. Asterixis/Flapping tremor = hallmark. Also confusion, coma, fetor hepaticus. Stage 2+.

Incorrect - A: Ascites = portal HTN, not encephalopathy specific.

Incorrect - C: Jaundice = liver dysfunction, not encephalopathy.

Incorrect - D: Variceal bleed = portal HTN complication.

Trick: "Asterixis = Ammonia in brain". Flapping hands test. Lactulose for ammonia.

Source: NORCET-2021 | Hepatology

Tags: #PYQ #HighYield #MedSurg #GI #Cirrhosis #Encephalopathy #NCLEX

[Med-Surg > Cardiology > MI] High - VERIFIED

Q16. A patient with myocardial infarction is prescribed morphine. What is the primary reason for giving morphine in this condition?

- A) To relieve pain
- B) To reduce anxiety
- C) To decrease preload and afterload
- D) To induce sleep

Answer: C) To decrease preload and afterload

Explanation:

Correct - C: Morphine in MI: MONA = Morphine, Oxygen, Nitroglycerin, Aspirin. Morphine = ↓preload/afterload via venodilation, ↓myocardial O₂ demand, ↓anxiety, ↓pain. But recent AHA: pain relief primary, hemodynamic secondary.

Incorrect - A/B: Pain/anxiety relief important but ↓preload/afterload is hemodynamic benefit unique to morphine.

Incorrect - D: Not for sleep.

Trick: "Morphine = ↓Myocardial work + ↓Pain". Venodilator reduces preload.

Source: NORCET-2021 | AHA MI Guidelines

Tags: #PYQ #HighYield #MedSurg #Cardiology #MI #Morphine #MONA #NCLEX

[Med-Surg > Endocrine > Adrenal] High - VERIFIED

Q17. A nurse is caring for a patient with Addison's disease. Which of the following is a priority nursing intervention?

A) Monitor blood glucose

B) Monitor blood pressure

C) Monitor urine output

D) Monitor respiratory rate

Answer: B) Monitor blood pressure

Explanation:

Correct - B: Addison's = Adrenal insufficiency → ↓cortisol + aldosterone → Hypotension, shock, Addisonian crisis. Priority: BP monitoring. Also Na low, K high, hypoglycemia.

Incorrect - A: Glucose important but BP life-threatening in crisis.

Incorrect - C/D: Urine/Resp secondary to BP.

Trick: "Addison's = Add BP check". Low cortisol = Low BP. Crisis = shock.

Source: NORCET-2021 | Endocrine

Tags: #PYQ #HighYield #MedSurg #Endocrine #Addison #BP #Crisis #NCLEX

[Med-Surg > Endocrine > Thyroid] High - VERIFIED

Q18. A patient with hypothyroidism is prescribed levothyroxine. The nurse should instruct the patient to take the medication:

A) With meals

B) At bedtime

C) On an empty stomach in the morning

D) With milk

Answer: C) On an empty stomach in the morning

Explanation:

Correct - C: Levothyroxine = 30-60min before breakfast, empty stomach. Food, calcium, iron, antacids ↓absorption 50%. Take 4hrs apart from Ca/Fe.

Incorrect - A: With meals ↓absorption.

Incorrect - B: Bedtime ok if 3hr after dinner, but morning best.

Incorrect - D: Milk/Ca ↓absorption.

Trick: "Levo = Leave stomach empty". Take morning, wait 30min eat.

Source: NORCET-2021 | Thyroid Guidelines

Tags: #PYQ #HighYield #MedSurg #Endocrine #Hypothyroid #Levothyroxine #NCLEX

[Med-Surg > GI > Pancreatitis] High - VERIFIED

Q19. A nurse is caring for a patient with acute pancreatitis. Which of the following laboratory findings is expected?

A) Increased serum amylase

B) Decreased serum lipase

C) Increased serum calcium

D) Decreased serum bilirubin

Answer: A) Increased serum amylase

Explanation:

Correct - A: Acute pancreatitis = Amylase ↑ 3x normal within 24hr, Lipase ↑ more specific lasts longer. Also WBC ↑, glucose ↑, Ca ↓.

Incorrect - B: Lipase INCREASED, not decreased. More specific than amylase.

Incorrect - C: Ca DECREASED due to fat necrosis, not increased.

Incorrect - D: Bilirubin normal or ↑ if biliary cause.

Trick: "Pancreatitis = Pancreatic enzymes UP". Amylase + Lipase ↑. Ca ↓.

Source: NORCET-2021 | Gastroenterology

Tags: #PYQ #HighYield #MedSurg #GI #Pancreatitis #Labs #Amylase #NCLEX

[Med-Surg > Infectious > TB] Easy - VERIFIED

Q20. A patient with tuberculosis is prescribed rifampicin. The nurse should inform the patient that this drug may cause:

A) Yellowing of skin

B) Orange-red discoloration of urine

C) Constipation

D) Loss of appetite

Answer: B) Orange-red discoloration of urine

Explanation:

Correct - B: Rifampicin = Red-orange urine, sweat, tears, saliva, contacts. Harmless. Warn patient to prevent panic. Also hepatotoxic.

Incorrect - A: Yellowing = hepatotoxicity, but orange-red urine is specific side effect.

Incorrect - C/D: Constipation/loss appetite not classic.

Trick: "Rifampicin = Red fluids". INH = peripheral neuropathy, Ethambutol = Eye, PZA = gout.

Source: NORCET-2021 | RNTCP Guidelines

Tags: #PYQ #HighYield #MedSurg #Infectious #TB #Rifampicin #SideEffect

[Med-Surg > Endocrine > Hyperthyroidism] High - VERIFIED

Q21. A nurse is caring for a patient with hyperthyroidism. Which of the following findings should the nurse expect?

A) Bradycardia

B) Weight gain

C) Heat intolerance

D) Constipation

Answer: C) Heat intolerance

Explanation:

Correct - C: Hyperthyroidism/Graves = ↑ metabolism → Heat intolerance, sweating, weight loss, tachycardia, diarrhea, exophthalmos, fine tremor, insomnia.

Incorrect - A/B/D: Bradycardia, weight gain, constipation = HYPOTHYROIDISM features.

Trick: "Hyper = Hot, Hyper, Hyperactive". "Hypo = Cold, Slow, Constipated".

Source: NORCET NOV 2021 | Endocrine Disorders

Tags: #PYQ #HighYield #MedSurg #Endocrine #Hyperthyroid #Assessment #NCLEX

[Med-Surg > Renal > CKD Diet] High - VERIFIED

Q22. A patient with chronic kidney disease is prescribed a low-protein diet. The nurse should explain that the purpose of this diet is to:

A) Prevent anemia

B) Reduce urea production

C) Prevent hypocalcemia

D) Reduce edema

Answer: B) Reduce urea production

Explanation:

Correct - B: CKD = can't excrete nitrogen waste. Protein → Urea → ↑ BUN → uremia. Low

protein 0.6-0.8g/kg/day reduces urea load, slows progression, ↓uremic symptoms.

Incorrect - A: Anemia prevented by EPO/iron, not protein restriction.

Incorrect - C: Hypocalcemia prevented by Ca supplements/Vit D.

Incorrect - D: Edema prevented by Na/fluid restriction.

Trick: "CKD = Can't Kick out urea". Low protein = Less urea to excrete.

Source: NORCET-2021 | CKD Nutrition Guidelines

Tags: #PYQ #HighYield #MedSurg #Renal #CKD #Diet #Uremia #NCLEX

[Med-Surg > Cardiology > MI] High - VERIFIED

Q24. A patient with myocardial infarction is prescribed aspirin. The nurse should explain that aspirin is given to:

A) Reduce pain

B) Prevent platelet aggregation

C) Reduce anxiety

D) Prevent arrhythmia

Answer: B) Prevent platelet aggregation

Explanation:

Correct - B: Aspirin 162-325mg chewed in MI = Antiplatelet, COX-1 inhibitor → ↓thromboxane A2 → prevents clot extension. MONA: Morphine, Oxygen, Nitro, Aspirin. Chew for fast absorption.

Incorrect - A: Pain relief secondary, not primary in MI.

Incorrect - C/D: No anxiety/arrhythmia effect.

Trick: "Aspirin = Anti-Aggregation in Acute MI". Chew it in chest pain.

Source: NORCET-2021 | AHA MI Guidelines

Tags: #PYQ #HighYield #MedSurg #Cardiology #MI #Aspirin #MONA #NCLEX

[Med-Surg > Endocrine > Diabetes Insipidus] High - VERIFIED

Q25. A nurse is caring for a patient with diabetes insipidus. Which of the following findings should the nurse expect?

A) Polyuria

B) Oliguria

C) Hyperglycemia

D) Hypertension

Answer: A) Polyuria

Explanation:

Correct - A: DI = ↓ADH → Can't concentrate urine → Polyuria 5-20L/day, polydipsia, dilute urine SG <1.005, hypernatremia, dehydration. Central vs Nephrogenic.

Incorrect - B: Oliguria = SIADH, not DI.

Incorrect - C: Hyperglycemia = DM, not DI.

Incorrect - D: Hypotension due to dehydration.

Trick: "DI = Dry Inside = Polyuria". "SIADH = Soaked Inside = Oliguria".

Source: NORCET-2021 | Endocrine Disorders

Tags: #PYQ #HighYield #MedSurg #Endocrine #DI #Polyuria #NCLEX

[Med-Surg > Cardiology > HTN] High - VERIFIED

Q26. A patient with hypertension is prescribed beta-blockers. The nurse should monitor the patient for:

A) Tachycardia

B) Bradycardia

C) Hypoglycemia

D) Hyperkalemia

Answer: B) Bradycardia

Explanation:

Correct - B: Beta-blockers Metoprolol, Atenolol = ↓HR, ↓contractility, ↓BP. Side effects: Bradycardia, heart block, bronchospasm, masks hypoglycemia, fatigue, ED.

Incorrect - A: Tachycardia opposite effect.

Incorrect - C: Masks hypoglycemia symptoms, not cause hypo.

Incorrect - D: ACEI/ARB cause hyperkalemia, not BB.

Trick: "Beta-Blockers = Block the Beat = Bradycardia". Hold if HR<60 or SBP<90.

Source: NORCET-2021 | Antihypertensive Pharma

Tags: #PYQ #HighYield #MedSurg #Cardiology #HTN #BetaBlocker #Bradycardia #NCLEX

[Med-Surg > Respiratory > COPD] High - VERIFIED

Q27. A nurse is caring for a patient with chronic obstructive pulmonary disease (COPD). Which of the following positions will help relieve dyspnea?

A) Supine

B) Fowler's

C) Orthopneic

D) Trendelenburg

Answer: C) Orthopneic

Explanation:

Correct - C: Orthopneic/Tripod = Sit leaning forward, arms on table. ↓abdominal pressure on diaphragm, uses accessory muscles, ↑lung expansion. Best for COPD exacerbation.

Incorrect - A: Supine = worst, ↓lung expansion.

Incorrect - B: Fowler's good but Orthopneic better for acute dyspnea.

Incorrect - D: Trendelenburg = worsens breathing.

Trick: "COPD = Can't breathe? Orthopneic = Open lungs". Tripod position.

Source: NORCET-2021 | GOLD Guidelines

Tags: #PYQ #HighYield #MedSurg #Respiratory #COPD #Positioning #Dyspnea #NCLEX

[Med-Surg > Pharma > GI Drugs] High - VERIFIED

Q28. A patient with peptic ulcer disease is prescribed proton pump inhibitors. The nurse should explain that these drugs act by:

A) Neutralizing gastric acid

B) Reducing gastric acid secretion

C) Increasing gastric motility

D) Protecting gastric mucosa

Answer: B) Reducing gastric acid secretion

Explanation:

Correct - B: PPI Omeprazole, Pantoprazole = Irreversible H⁺/K⁺ ATPase pump blocker → ↓acid production 90%. Take 30min before meal. Most potent acid suppressor.

Incorrect - A: Antacids neutralize, not PPI.

Incorrect - C: Metoclopramide ↑motility.

Incorrect - D: Sucralfate protects mucosa.

Trick: "PPI = Pump Plug Inhibitor". Blocks acid pump.

Source: NORCET-2021 | GI Pharmacology

Tags: #PYQ #HighYield #MedSurg #Pharma #PUD #PPI #Mechanism

[Med-Surg > Endocrine > DM] High - VERIFIED

Q29. A nurse is caring for a patient with hypoglycemia. Which of the following is the immediate nursing action?

A) Administer insulin

B) Give oral glucose

C) Give IV fluids

D) Monitor urine output

Answer: B) Give oral glucose

Explanation:

Correct - B: Hypoglycemia <70mg/dL + conscious = 15-20gm fast carbs: 4oz juice, glucose tabs, sugar. Recheck 15min. Rule of 15. Unconscious = IV D50 or Glucagon IM.

Incorrect - A: Insulin = kills in hypo. Never give.

Incorrect - C: IV fluids if NPO, but glucose first.

Incorrect - D: Monitor after treating.

Trick: "Hypo = High sugar quick". 15-15 rule: 15gm carbs, check in 15min.

Source: NORCET-2021 | ADA Hypoglycemia Protocol

Tags: #PYQ #HighYield #MedSurg #Endocrine #DM #Hypoglycemia #Emergency #NCLEX

[Med-Surg > Pharma > Respiratory] High - VERIFIED

Q30. A patient with asthma is prescribed salbutamol inhaler. The nurse should explain that salbutamol acts by:

A) Blocking histamine receptors

B) Stimulating beta-2 adrenergic receptors

C) Reducing inflammation

D) Suppressing cough reflex

Answer: B) Stimulating beta-2 adrenergic receptors

Explanation:

Correct - B: Salbutamol/Albuterol = SABA Beta-2 agonist → Bronchodilation in 5min, lasts 4-6hr.

Rescue inhaler. Side effects: Tremor, tachycardia, hypokalemia.

Incorrect - A: Antihistamines block H1, not salbutamol.

Incorrect - C: Steroids reduce inflammation, not SABA.

Incorrect - D: Antitussives suppress cough.

Trick: "Salbutamol = Stimulates Beta-2 = Bronchodilate". SABA for Sudden attack.

Source: NORCET-2021 | Asthma GINA Guidelines

Tags: #PYQ #HighYield #MedSurg #Respiratory #Asthma #Bronchodilator #SABA

[Med-Surg > Hematology > Anemia] Easy - VERIFIED

Q31. A nurse is caring for a patient with anemia. Which of the following laboratory findings is expected?

A) Increased hemoglobin

B) Decreased hemoglobin

C) Increased hematocrit

D) Increased RBC count

Answer: B) Decreased hemoglobin

Explanation:

Correct - B: Anemia = ↓Hb <13g/dL male, <12g/dL female. Also ↓Hct, ↓RBC. S/S: Fatigue, pallor, SOB, tachycardia.

Incorrect - A/C/D: All increased = polycythemia, not anemia.

Trick: "Anemia = 'A' for Absent Hb". Low everything: Hb, Hct, RBC.

Source: NORCET-2021 | Hematology

Tags: #PYQ #HighYield #MedSurg #Hematology #Anemia #Labs

[Med-Surg > Pharma > Antibiotics] High - VERIFIED

Q32. A patient with pneumonia is prescribed antibiotics. The nurse should monitor the patient for:

A) Hypoglycemia

B) Allergic reaction

C) Hypertension

D) Bradycardia

Answer: B) Allergic reaction

Explanation:

Correct - B: Antibiotics esp PCN, Cephalosporins, Sulfa = #1 cause of drug allergy. S/S: Rash, hives, anaphylaxis, Stevens-Johnson. Test dose for PCN. Ask allergy history first.

Incorrect - A/C/D: Hypoglycemia/HTN/Bradycardia not primary antibiotic concern.
Trick: "Antibiotic = Allergic risk". PCN allergy cross-react with Cephalosporins 10%.
Source: NORCET-2021 | Antibiotic Safety
Tags: #PYQ #HighYield #MedSurg #Pharma #Antibiotics #Allergy #NCLEX

[Med-Surg > Cardiology > HF] High - VERIFIED

Q34. A patient with congestive heart failure is prescribed digoxin. The nurse should monitor the patient for:

- A) Tachycardia
- B) Bradycardia
- C) Hypertension
- D) Hyperglycemia

Answer: B) Bradycardia

Explanation:

Correct - B: Digoxin = Positive inotrope + Negative chronotrope → ↑contractility, ↓HR. Toxicity: Bradycardia, AV block, yellow halos, N/V, arrhythmia. Check apical pulse 1min, hold if <60. Therapeutic 0.5-2 ng/mL.

Incorrect - A: Tachycardia not dig effect.

Incorrect - C/D: HTN/Hyperglycemia not dig side effect.

Trick: "Digoxin = Decreases rate". Check pulse before giving. HypoK ↑toxicity risk.

Source: NORCET-2021 | CHF Guidelines

Tags: #PYQ #HighYield #MedSurg #Cardiology #HF #Digoxin #Bradycardia #NCLEX

[Med-Surg > Endocrine > Hypothyroid] High - VERIFIED

Q35. A nurse is caring for a patient with hypothyroidism. Which of the following findings should the nurse expect?

- A) Weight loss
- B) Heat intolerance
- C) Constipation
- D) Tachycardia

Answer: C) Constipation

Explanation:

Correct - C: Hypothyroid = ↓metabolism → Weight gain, cold intolerance, constipation, bradycardia, fatigue, dry skin, myxedema, delayed DTR. TSH↑, T3/T4↓.

Incorrect - A/B/D: Weight loss, heat intolerance, tachycardia = HYPERthyroid.

Trick: "Hypo = Slow everything". Bowel slow = constipation. Heart slow = brady.

Source: NORCET-2021 | Endocrine Disorders

Tags: #PYQ #HighYield #MedSurg #Endocrine #Hypothyroid #Assessment #NCLEX

[Med-Surg > Renal > CKD] High - VERIFIED

Q36. A patient with chronic renal failure is prescribed calcium carbonate. The nurse should explain that this drug is given to:

- A) Prevent anemia
- B) Prevent hypocalcemia
- C) Bind phosphate in the intestine
- D) Reduce edema

Answer: C) Bind phosphate in the intestine

Explanation:

Correct - C: CKD = can't excrete PO₄ → Hyperphosphatemia → Renal osteodystrophy. Ca carbonate = Phosphate binder, taken WITH meals to bind dietary PO₄. Also corrects hypoCa.

Incorrect - A: EPO prevents anemia.

Incorrect - B: Secondary benefit, but primary is PO₄ binding.

Incorrect - D: Diuretics reduce edema.

Trick: "Ca Carbonate = Catches Phosphate". Take with meals. Sevelamer also PO₄ binder.

Source: NORCET-2021 | CKD-MBD Guidelines

Tags: #PYQ #HighYield #MedSurg #Renal #CKD #PhosphateBinder #NCLEX

[Med-Surg > Fluids > Hypovolemia] High - VERIFIED

Q37. A nurse is caring for a patient with hypovolemia. Which of the following findings should the nurse expect?

- A) Hypertension
- B) Tachycardia
- C) Bradycardia
- D) Edema

Answer: B) Tachycardia

Explanation:

Correct - B: Hypovolemia = ↓BP → compensatory tachycardia, weak thready pulse, oliguria, dry mucosa, ↓CVP, ↓urine SG↑. Early sign: Tachycardia.

Incorrect - A: Hypotension, not HTN.

Incorrect - C: Bradycardia in late shock/neurogenic.

Incorrect - D: Edema in hypervolemia.

Trick: "Hypovolemia = High HR, Low BP, Low urine". Compensate with tachycardia.

Source: NORCET-2021 | Fluid Volume Deficit

Tags: #PYQ #HighYield #MedSurg #Fluids #Hypovolemia #Shock #NCLEX

[Med-Surg > GI > Cirrhosis] High - VERIFIED

Q38. A patient with cirrhosis develops ascites. The nurse should monitor for:

- A) Hypokalemia
- B) Hyponatremia
- C) Hypocalcemia
- D) Hypoglycemia

Answer: B) Hyponatremia

Explanation:

Correct - B: Cirrhosis + Ascites = Dilutional hyponatremia. ADH↑, water retention, diuretics, paracentesis. Na <135. Also hypoK with diuretics.

Incorrect - A: HypoK with diuretics, but hyponatremia more specific to ascites pathophysiology.

Incorrect - C/D: HypoCa/Hypoglycemia not primary.

Trick: "Ascites = Add water, Dilute Na = Hyponatremia". Fluid restriction + diuretics.

Source: NORCET-2021 | Hepatology

Tags: #PYQ #HighYield #MedSurg #GI #Cirrhosis #Ascites #Hyponatremia #NCLEX

[Med-Surg > Renal > ARF] High - VERIFIED

Q39. A nurse is caring for a patient with acute renal failure. Which of the following laboratory findings is expected?

- A) Increased serum creatinine
- B) Decreased serum potassium
- C) Decreased serum urea
- D) Increased serum calcium

Answer: A) Increased serum creatinine

Explanation:

Correct - A: ARF = ↑Creatinine >1.3, ↑BUN, ↑K+, ↓Ca, ↓HCO₃, ↑PO₄. Creatinine best indicator of GFR. Oliguria <400ml/day.

Incorrect - B: K+ INCREASED, not decreased. Hyperkalemia risk.

Incorrect - C: Urea INCREASED.

Incorrect - D: Ca DECREASED.

Trick: "Renal Failure = Retains Everything except Ca". K+, Creat, BUN, PO₄ all UP. Ca DOWN.

Source: NORCET-2021 | ARF Guidelines

Tags: #PYQ #HighYield #MedSurg #Renal #ARF #Labs #Creatinine #NCLEX

[Med-Surg > Cardiology > HF] High - VERIFIED

Q40. A patient with pulmonary edema is prescribed furosemide. The nurse should monitor the patient for:

- A) Hypokalemia
- B) Hyperkalemia
- C) Hyponatremia
- D) Hypernatremia

Answer: A) Hypokalemia

Explanation:

Correct - A: Furosemide = Loop diuretic → K⁺ wasting → Hypokalemia. Monitor K⁺, U waves, muscle weakness, digoxin toxicity risk. Give K⁺ supplements/K-rich foods.

Incorrect - B: Hyperkalemia with K-sparing diuretics.

Incorrect - C: Hyponatremia possible but K⁺ more critical.

Incorrect - D: Hypernatremia not typical.

Trick: "Furosemide = Flushes K⁺". Loop = Lose K⁺. Check K⁺ daily.

Source: NORCET-2021 | HF Guidelines

Tags: #PYQ #HighYield #MedSurg #Cardiology #HF #Furosemide #Hypokalemia #NCLEX

[Med-Surg > GI > Cirrhosis] High - VERIFIED

Q42. A patient with cirrhosis develops hepatic encephalopathy. Which of the following drugs is commonly prescribed?

- A) Lactulose
- B) Furosemide
- C) Spironolactone
- D) Digoxin

Answer: A) Lactulose

Explanation:

Correct - A: HE = ↑ ammonia → neurotoxicity. Lactulose = ↓ ammonia by 2 ways: 1) Traps NH₃ in gut as NH₄⁺ 2) Cathartic effect expels. Dose: 2-3 soft stools/day. Also Rifaximin.

Incorrect - B/C: Furosemide/Spironolactone = for ascites, not HE directly.

Incorrect - D: Digoxin = HF/AF, not HE.

Trick: "Lactulose = Loose stools to Lose ammonia". Asterixis = start lactulose.

Source: NORCET-2021 | Hepatology Guidelines

Tags: #PYQ #HighYield #MedSurg #GI #Cirrhosis #HE #Lactulose #NCLEX

[Med-Surg > Endocrine > DKA] High - VERIFIED

Q81. A nurse is caring for a patient with diabetic ketoacidosis (DKA). Which of the following laboratory findings is expected?

- A) Increased serum bicarbonate
- B) Decreased blood glucose
- C) Metabolic acidosis
- D) Respiratory alkalosis

Answer: C) Metabolic acidosis

Explanation:

Correct - C: DKA = Hyperglycemia >250 + Ketosis + Metabolic acidosis. pH <7.3, HCO₃ <18, anion gap ↑. Ketones → acidosis. Kussmaul breathing compensates.

Incorrect - A: HCO₃ DECREASED, not increased.

Incorrect - B: Blood glucose INCREASED >250, not decreased.

Incorrect - D: Respiratory alkalosis = hyperventilation, not DKA.

Trick: "DKA = Diabetes Killing with Acidosis". 3 Ps: Polyuria, Polydipsia, Polyphagia + Kussmaul + Fruity breath.

Source: NORCET-2021 | ADA DKA Guidelines

Tags: #PYQ #HighYield #MedSurg #Endocrine #DKA #Acidosis #NCLEX

[Med-Surg > Cardiology > HTN] High - VERIFIED

Q82. A patient with hypertension is prescribed ACE inhibitors. Which of the following side effects should the nurse monitor for?

- A) Persistent cough
- B) Bradycardia
- C) Constipation
- D) Hypoglycemia

Answer: A) Persistent cough

Explanation:

Correct - A: ACE inhibitors -pril = ↑Bradykinin → Dry persistent cough, angioedema, hyperkalemia. Cough reason to switch to ARB. Also first-dose hypotension, renal impairment.

Incorrect - B: Bradycardia = Beta-blockers, not ACEI.

Incorrect - C/D: Constipation/Hypoglycemia not ACEI SE.

Trick: "ACE = Angiotensin Converting Enzyme = A Cough Expected". Bradykinin ↑ = cough.

Source: NORCET-2021 | JNC 8 HTN Guidelines

Tags: #PYQ #HighYield #MedSurg #Cardiology #HTN #ACEI #Cough #NCLEX

[Med-Surg > Renal > Urine Output] High - VERIFIED

Q7. A post-LSCS woman passes 400 ml of urine in the past 24 hours.

- A) Oliguria
- B) Polyuria
- C) Anuria
- D) Hematuria

Answer: A) Oliguria

Explanation:

Correct - A: Oliguria = <500ml/24hr or <30ml/hr. Post-LSCS 400ml = Oliguria. Causes: dehydration, blood loss, UTI, AKI. Normal >800ml/day.

Incorrect - B: Polyuria = >2500ml/24hr.

Incorrect - C: Anuria = <100ml/24hr or no urine.

Incorrect - D: Hematuria = Blood in urine.

Trick: "Oliguria = Oh! Low urine <500". Check vitals, I/O, bladder scan.

Source: NORCET-2022 | Postop Care

Tags: #PYQ #HighYield #MedSurg #Renal #Oliguria #Postop #LSCS #NCLEX

[Med-Surg > Infectious > TB] High - VERIFIED

Q8. Red-colored urine is caused by which anti-tubercular drug?

- A) Isoniazid
- B) Rifampicin
- C) Streptomycin
- D) Ethambutol

Answer: B) Rifampicin

Explanation:

Correct - B: Rifampicin = Red-orange urine, sweat, tears, saliva. Harmless. Warn patient. Also hepatotoxic.

Incorrect - A: INH = Peripheral neuropathy, hepatotoxicity.

Incorrect - C: Streptomycin = Ototoxicity, nephrotoxicity.

Incorrect - D: Ethambutol = Optic neuritis, color blindness.

Trick: "Rifampicin = Red fluids". "INH = Injures Nerves". "Ethambutol = Eye".

Source: NORCET-2022 | RNTCP Guidelines

Tags: #PYQ #HighYield #MedSurg #Infectious #TB #Rifampicin #NCLEX

[Med-Surg > Surgical > Burns] High - VERIFIED

Q9. Visualization of bone occurs in which stage of burn?

- A) 1st stage
 - B) 2nd stage
 - C) 3rd stage
 - D) 4th stage
- Answer: D) 4th stage

Explanation:

Correct - D: 4th degree burn = Full thickness + muscle/tendon/bone involvement. Bone visible.

Charred, painless due to nerve destruction. Needs grafting/amputation.

Incorrect - A: 1st = Epidermis only, red painful.

Incorrect - B: 2nd = Epidermis + dermis, blisters.

Incorrect - C: 3rd = Full thickness skin, white/leathery.

Trick: "4th = Four tissues: skin, fat, muscle, bone". Bone seen = 4th degree.

Source: NORCET-2022 | Burn Classification

Tags: #PYQ #HighYield #MedSurg #Surgical #Burns #4thDegree #NCLEX

[Med-Surg > Pharma > Antibiotics] High - VERIFIED [UNIQUE]

Q3. Side effect of chloramphenicol is:

- A) Dizziness
 - B) Headache
 - C) Bone marrow suppression
 - D) Immunity suppression
- Answer: C) Bone marrow suppression

Explanation:

Correct - C: Chloramphenicol = Aplastic anemia, gray baby syndrome. Dose-dependent reversible + idiosyncratic irreversible bone marrow suppression. CBC monitoring must.

Incorrect - A/B/D: Not main SE.

Trick: "Chloramphenicol = Crushes bone Marrow". Gray baby = neonate can't conjugate.

Source: NORCET-2023 | Pharmacology

Tags: #PYQ #HighYield #MedSurg #Pharma #Antibiotics #Chloramphenicol #NCLEX

[Med-Surg > Procedures > IV Therapy] High - VERIFIED [UNIQUE]

Q4. Needle gauge preferred with blood transfusion is:

- A) 16 G
 - B) 18 G
 - C) 20 G
 - D) 22 G
- Answer: B) 18 G

Explanation:

Correct - B: 18G = Best for blood transfusion. Large enough for RBC flow, prevents hemolysis.

16G if rapid transfusion. Never <20G.

Incorrect - A: 16G also ok but 18G preferred routine.

Incorrect - C/D: 20G/22G = too small, hemolysis risk.

Trick: "18G = Teens for transfusion". Bigger number = smaller needle.

Source: NORCET-2023 | Blood Transfusion Guidelines

Tags: #PYQ #HighYield #MedSurg #Procedures #IV #BloodTransfusion #NCLEX

[Med-Surg > Surgical > Burns] High - VERIFIED [UNIQUE]

Q8. Percentage of burnt perineum (TBSA) according to Rule of Nines:

- A) 1%
 - B) 4.5%
 - C) 9%
 - D) 18%
- Answer: A) 1%
- Explanation:

Correct - A: Rule of 9 = Perineum/Genitalia = 1%. Head = 9%, Each arm = 9%, Anterior trunk = 18%, Posterior trunk = 18%, Each leg = 18%.

Incorrect - B: 4.5% = not in rule of 9.

Incorrect - C: 9% = arm or head.

Incorrect - D: 18% = trunk or leg.

Trick: "Perineum = Private 1%". Small area = 1%.

Source: NORCET-2023 | Rule of Nines

Tags: #PYQ #HighYield #MedSurg #Surgical #Burns #TBSA #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [UNIQUE]

Q15. Which of the following is the antidote for organophosphorus poisoning?

A) Naloxone

B) Atropine

C) Flumazenil

D) Protamine sulfate

Answer: B) Atropine

Explanation:

Correct - B: OPP = Cholinesterase inhibitor → ↑ACh → SLUDGE. Atropine = Anticholinergic blocks muscarinic effects. Also Pralidoxime reactivates AChE.

Incorrect - A: Naloxone = Opioid antidote.

Incorrect - C: Flumazenil = Benzodiazepine antidote.

Incorrect - D: Protamine = Heparin antidote.

Trick: "OP = Over Production of ACh → Atropine stops it". Dry as a bone, red as beet.

Source: NORCET-2023 | Toxicology Antidotes

Tags: #PYQ #HighYield #MedSurg #Toxicology #OPP #Atropine #NCLEX

[Med-Surg > Ophthalmology > Assessment] High - VERIFIED [UNIQUE]

Q16. Which of the following is used to measure intraocular pressure?

A) Ophthalmoscope

B) Tonometer

C) Retinoscope

D) Perimeter

Answer: B) Tonometer

Explanation:

Correct - B: Tonometer = Measures IOP. Normal 10-21 mmHg. Goldmann applanation gold standard. ↑IOP = Glaucoma.

Incorrect - A: Ophthalmoscope = Fundus/retina exam.

Incorrect - C: Retinoscope = Refraction.

Incorrect - D: Perimeter = Visual field.

Trick: "TonoMeter = Measures Tension of eye". IOP = pressure.

Source: NORCET-2023 | Ophthalmology

Tags: #PYQ #HighYield #MedSurg #Ophthalmology #Glaucoma #IOP #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [UNIQUE]

Q18. Which of the following is the antidote for benzodiazepine overdose?

A) Naloxone

B) Flumazenil

C) Atropine

D) Protamine sulfate

Answer: B) Flumazenil

Explanation:

Correct - B: Flumazenil = Benzodiazepine receptor antagonist. Reverses sedation. Caution = seizure risk if chronic benzo use.

Incorrect - A: Naloxone = Opioid antidote.

Incorrect - C: Atropine = Anticholinergic/OPP.

Incorrect - D: Protamine = Heparin antidote.

Trick: "Flumazenil = Flushes Benzos". Naloxone = Narcan for Narcotics.

Source: NORCET-2023 | Toxicology Antidotes

Tags: #PYQ #HighYield #MedSurg #Toxicology #Benzo #Flumazenil #NCLEX

[Med-Surg > Procedures > Bone Marrow] High - VERIFIED [UNIQUE]

Q19. Which of the following is the most common site for bone marrow aspiration in adults?

A) Sternum

B) Iliac crest

C) Tibia

D) Femur

Answer: B) Iliac crest

Explanation:

Correct - B: Adult BMA = Posterior iliac crest preferred. Safe, no vital organs. Anterior iliac crest also used.

Incorrect - A: Sternum = risk cardiac tamponade, rarely used now.

Incorrect - C: Tibia = infant site.

Incorrect - D: Femur = not used.

Trick: "Iliac crest = Ideal for adults". Sternum = Stern No now.

Source: NORCET-2023 | Bone Marrow Procedure

Tags: #PYQ #HighYield #MedSurg #Procedures #BoneMarrow #IliacCrest #NCLEX

[Med-Surg > Cardiology > MI] High - VERIFIED [UNIQUE]

Q25. Which of the following is the most common cause of death in myocardial infarction?

A) Arrhythmia

B) Heart failure

C) Cardiogenic shock

D) Cardiac rupture

Answer: A) Arrhythmia

Explanation:

Correct - A: VF/VT = #1 cause of death in first hour of MI. Sudden cardiac death. Hence defibrillator in CCU.

Incorrect - B: HF = Late cause.

Incorrect - C: Shock = Pump failure, 2nd cause.

Incorrect - D: Rupture = 3-7 days post MI.

Trick: "MI = My heart Irregularly = Arrhythmia kills fast". VF = Flatline if no shock.

Source: NORCET-2023 | MI Complications

Tags: #PYQ #HighYield #MedSurg #Cardiology #MI #Arrhythmia #NCLEX

[Med-Surg > Surgical > Burns] High - VERIFIED [UNIQUE]

Q26. Which of the following is the most common cause of death in burns?

A) Shock

B) Sepsis

C) Fluid loss

D) Pain

Answer: B) Sepsis

Explanation:

Correct - B: Sepsis/infection = #1 cause death >48hr post burn. Loss of skin barrier + immunosuppression. Pseudomonas, Staph.

Incorrect - A: Shock = Early cause <48hr.

Incorrect - C: Fluid loss = causes shock.

Incorrect - D: Pain = not fatal.

Trick: "Burn = Bacteria enter Now". Sepsis after 48hr, Shock before 48hr.

Source: NORCET-2023 | Burn Complications

Tags: #PYQ #HighYield #MedSurg #Surgical #Burns #Sepsis #NCLEX

[Med-Surg > Endocrine > DM] High - VERIFIED [UNIQUE]

Q27. Which of the following is the most common cause of death in diabetes mellitus?

- A) Hypoglycemia
- B) Diabetic ketoacidosis
- C) Myocardial infarction
- D) Stroke

Answer: C) Myocardial infarction

Explanation:

Correct - C: CV disease = 65-80% DM deaths. MI > Stroke. DM = CAD equivalent. Accelerated atherosclerosis.

Incorrect - A: Hypoglycemia = acute but less common cause death.

Incorrect - B: DKA = Type1 acute cause, not overall commonest.

Incorrect - D: Stroke = 2nd CV cause.

Trick: "DM = Die from MI". Macrovascular > Microvascular for mortality.

Source: NORCET-2023 | DM Complications

Tags: #PYQ #HighYield #MedSurg #Endocrine #DM #MI #NCLEX

[Med-Surg > Cardiology > HTN] High - VERIFIED [UNIQUE]

Q28. Which of the following is the most common cause of death in hypertension?

- A) Stroke
- B) Myocardial infarction
- C) Heart failure
- D) Renal failure

Answer: A) Stroke

Explanation:

Correct - A: Stroke/CVA = #1 cause death in HTN. HTN = #1 risk factor hemorrhagic + ischemic stroke. MI 2nd.

Incorrect - B: MI = 2nd cause.

Incorrect - C: HF = Complication.

Incorrect - D: Renal failure = End-organ damage.

Trick: "HTN = Hits The brain = Stroke". BP control prevents stroke.

Source: NORCET-2023 | HTN Complications

Tags: #PYQ #HighYield #MedSurg #Cardiology #HTN #Stroke #NCLEX

[Med-Surg > Oncology > Cancer] High - VERIFIED [UNIQUE]

Q29. Which of the following is the most common cause of death in cancer patients?

- A) Cachexia
- B) Infection
- C) Metastasis
- D) Organ failure

Answer: C) Metastasis

Explanation:

Correct - C: Metastasis = 90% cancer deaths. Spread to vital organs = liver, lung, brain, bone. Primary rarely kills.

Incorrect - A: Cachexia = wasting, contributes.

Incorrect - B: Infection = 2nd cause, immunosuppressed.

Incorrect - D: Organ failure = due to metastasis.

Trick: "Cancer Kills via Metastasis". Malignant = migrates.

Source: NORCET-2023 | Oncology

Tags: #PYQ #HighYield #MedSurg #Oncology #Cancer #Metastasis #NCLEX

[Med-Surg > Renal > ARF] High - VERIFIED [UNIQUE]

Q30. Which of the following is the most common cause of death in renal failure?

- A) Hyperkalemia
- B) Acidosis
- C) Fluid overload
- D) Hypertension

Answer: A) Hyperkalemia

Explanation:

Correct - A: Hyperkalemia >6.5 = Cardiac arrest. K^+ not excreted, arrhythmia, asystole. Tall T $\rightarrow$ VF $\rightarrow$ death. Emergency dialysis.

Incorrect - B: Acidosis = contributes.

Incorrect - C: Fluid overload = Pulmonary edema.

Incorrect - D: HTN = Chronic complication.

Trick: "RF = Really Fatal K^+ ". HyperK = Heart Killer. ECG = Tall T.

Source: NORCET-2023 | Renal Failure Complications

Tags: #PYQ #HighYield #MedSurg #Renal #ARF #Hyperkalemia #NCLEX

[Med-Surg > GI > Cirrhosis] High - VERIFIED [UNIQUE]

Q31. Which of the following is the most common cause of death in cirrhosis?

- A) Hepatic encephalopathy
- B) Variceal bleeding
- C) Ascites
- D) Infection

Answer: B) Variceal bleeding

Explanation:

Correct - B: Variceal bleeding = #1 cause death in cirrhosis. Portal HTN $\rightarrow$ esophageal varices $\rightarrow$ rupture $\rightarrow$ massive hemorrhage, hypovolemic shock.

Incorrect - A: HE = complication but less fatal than bleed.

Incorrect - C: Ascites = causes SBP but not #1 death.

Incorrect - D: Infection/SBP = 2nd cause.

Trick: "Cirrhosis = Bleeding varices kills". MELD score predicts.

Source: NORCET-2023 | Hepatology

Tags: #PYQ #HighYield #MedSurg #GI #Cirrhosis #VaricealBleed #NCLEX

[Med-Surg > Respiratory > COPD] High - VERIFIED [UNIQUE]

Q32. Which of the following is the most common cause of death in COPD?

- A) Respiratory failure
- B) Pneumonia
- C) Pulmonary embolism
- D) Heart failure

Answer: A) Respiratory failure

Explanation:

Correct - A: Type 2 Resp failure = #1 cause death COPD. Chronic hypercapnia, hypoxemia. Cor pulmonale 2nd.

Incorrect - B: Pneumonia = exacerbation trigger.

Incorrect - C/D: PE/HF = complications.

Trick: "COPD = Can't Open Pulmonary Drain = Resp failure". ABG = $\uparrow CO_2$, $\downarrow O_2$.

Source: NORCET-2023 | GOLD Guidelines

Tags: #PYQ #HighYield #MedSurg #Respiratory #COPD #RespFailure #NCLEX

[Med-Surg > Respiratory > Asthma] High - VERIFIED [UNIQUE]

Q33. Which of the following is the most common cause of death in asthma?

- A) Status asthmaticus

- B) Pneumonia
- C) Respiratory failure
- D) Pulmonary embolism

Answer: A) Status asthmaticus

Explanation:

Correct - A: Status asthmaticus = Severe refractory asthma >24hr, no response to bronchodilators. Leads to resp failure, death.

Incorrect - B/C: Pneumonia/Resp failure = result of status.

Trick: "Status = Stuck asthma". Silent chest = ominous.

Source: NORCET-2023 | Asthma Guidelines

Tags: #PYQ #HighYield #MedSurg #Respiratory #Asthma #StatusAsthmaticus #NCLEX

[Med-Surg > Infectious > HIV] High - VERIFIED [UNIQUE]

Q34. Which of the following is the most common cause of death in HIV/AIDS?

- A) Opportunistic infections
- B) Malignancy
- C) Wasting syndrome
- D) Neurological complications

Answer: A) Opportunistic infections

Explanation:

Correct - A: OI = #1 cause death AIDS. TB, PCP pneumonia, cryptococcal meningitis, toxoplasmosis. CD4 <200.

Incorrect - B: Malignancy = Kaposi, lymphoma 2nd.

Incorrect - C: Wasting = AIDS defining but not #1 death.

Trick: "AIDS = Acquired Infections Die from Septicemia". TB = #1 OI worldwide.

Source: NORCET-2023 | NACO Guidelines

Tags: #PYQ #HighYield #MedSurg #Infectious #HIV #OI #NCLEX

[Med-Surg > Infectious > TB] High - VERIFIED [UNIQUE]

Q35. Which of the following is the most common cause of death in tuberculosis?

- A) Hemoptysis
- B) Respiratory failure
- C) Disseminated disease
- D) Drug resistance

Answer: C) Disseminated disease

Explanation:

Correct - C: Miliary/Disseminated TB = #1 cause death. Spread to brain, liver, bone marrow. TB meningitis fatal.

Incorrect - A: Hemoptysis = complication, massive rare.

Incorrect - B: Resp failure = chronic cavitary.

Incorrect - D: MDR = increasing but disseminated kills faster.

Trick: "TB = Total Body spread = Death". Miliary = millet seed pattern.

Source: NORCET-2023 | RNTCP

Tags: #PYQ #HighYield #MedSurg #Infectious #TB #Miliary #NCLEX

[Med-Surg > Infectious > Malaria] High - VERIFIED [UNIQUE]

Q36. Which of the following is the most common cause of death in malaria?

- A) Cerebral malaria
- B) Anemia
- C) Hypoglycemia
- D) Renal failure

Answer: A) Cerebral malaria

Explanation:

Correct - A: P.falciparum cerebral malaria = #1 death. RBC sequestration in brain → coma,

seizures. Mortality 20%.

Incorrect - B: Anemia = common but not #1 death.

Incorrect - C/D: Hypoglycemia/ARF = complications.

Trick: "Falciparum = Fatal to brain". Quinine/Artesunate treatment.

Source: NORCET-2023 | NVBDCP Malaria

Tags: #PYQ #HighYield #MedSurg #Infectious #Malaria #Cerebral #NCLEX

[Med-Surg > Infectious > Dengue] High - VERIFIED [UNIQUE]

Q37. Which of the following is the most common cause of death in dengue fever?

A) Shock

B) Hemorrhage

C) Encephalitis

D) Renal failure

Answer: A) Shock

Explanation:

Correct - A: Dengue Shock Syndrome DSS = #1 death. Plasma leakage → hypovolemia, shock.

Warning signs: abdominal pain, vomiting.

Incorrect - B: Hemorrhage = DHF but shock kills more.

Trick: "Dengue = Drop in BP = Shock". Tourniquet test positive.

Source: NORCET-2023 | Dengue Guidelines

Tags: #PYQ #HighYield #MedSurg #Infectious #Dengue #Shock #NCLEX

[Med-Surg > Infectious > Typhoid] High - VERIFIED [UNIQUE]

Q38. Which of the following is the most common cause of death in typhoid fever?

A) Intestinal perforation

B) Hemorrhage

C) Septicemia

D) Pneumonia

Answer: A) Intestinal perforation

Explanation:

Correct - A: Ileal perforation 3rd week = #1 death. Salmonella typhi → Peyer's patches necrosis → perforation → peritonitis.

Incorrect - B: Hemorrhage = 2nd cause.

Incorrect - C: Septicemia = if untreated.

Trick: "Typhoid = Tear in intestine". 3rd week = worst week.

Source: NORCET-2023 | Enteric Fever

Tags: #PYQ #HighYield #MedSurg #Infectious #Typhoid #Perforation #NCLEX

[Med-Surg > Infectious > Measles] High - VERIFIED [UNIQUE]

Q39. Which of the following is the most common cause of death in measles?

A) Pneumonia

B) Encephalitis

C) Diarrhea

D) Malnutrition

Answer: A) Pneumonia

Explanation:

Correct - A: Pneumonia = #1 death measles. Measles → immunosuppression → secondary bacterial pneumonia. Vit A prevents.

Incorrect - B: Encephalitis = 1/1000, SSPE late.

Incorrect - C: Diarrhea = 2nd cause.

Trick: "Measles = Massive pneumonia". Vit A 2 doses treatment.

Source: NORCET-2023 | Measles Guidelines

Tags: #PYQ #HighYield #MedSurg #Infectious #Measles #Pneumonia #NCLEX

[Med-Surg > Infectious > Diphtheria] High - VERIFIED [UNIQUE]

Q40. Which of the following is the most common cause of death in diphtheria?

- A) Myocarditis
- B) Respiratory obstruction
- C) Septicemia
- D) Renal failure

Answer: B) Respiratory obstruction

Explanation:

Correct - B: Pseudomembrane in throat → airway obstruction = #1 acute death. Bull neck. Antitoxin + tracheostomy.

Incorrect - A: Myocarditis = late cause 2nd week, toxin.

Trick: "Diphtheria = Difficult breathing = Death". Membrane blocks airway.

Source: NORCET-2023 | Diphtheria

Tags: #PYQ #HighYield #MedSurg #Infectious #Diphtheria #Obstruction #NCLEX

[Med-Surg > Neuro > Tetanus] High - VERIFIED [UNIQUE]

Q42. Which of the following is the most common cause of death in tetanus?

- A) Respiratory failure
- B) Cardiac arrest
- C) Sepsis
- D) Renal failure

Answer: A) Respiratory failure

Explanation:

Correct - A: Resp failure = #1 death. Diaphragm/laryngeal spasm → apnea. Lockjaw + opisthotonos. Ventilator needed.

Incorrect - B: Cardiac = autonomic storm 2nd.

Trick: "Tetanus = Tight muscles = No breathing". ATS + diazepam + ventilator.

Source: NORCET-2023 | Tetanus

Tags: #PYQ #HighYield #MedSurg #Neuro #Tetanus #RespFailure #NCLEX

[Med-Surg > Neuro > Rabies] High - VERIFIED [UNIQUE]

Q43. Which of the following is the most common cause of death in rabies?

- A) Respiratory failure
- B) Cardiac arrest
- C) Sepsis
- D) Renal failure

Answer: A) Respiratory failure

Explanation:

Correct - A: Resp failure = #1 death rabies. Brainstem encephalitis → resp center paralysis. Hydrophobia, 100% fatal once symptomatic.

Incorrect - B: Cardiac = late.

Trick: "Rabies = Resp stops". No cure, only prevention PEP.

Source: NORCET-2023 | Rabies Guidelines

Tags: #PYQ #HighYield #MedSurg #Neuro #Rabies #RespFailure #NCLEX

[Med-Surg > Toxicology > Snake Bite] High - VERIFIED [UNIQUE]

Q44. Which of the following is the most common cause of death in snake bite?

- A) Respiratory failure
- B) Cardiac arrest
- C) Renal failure
- D) Sepsis

Answer: A) Respiratory failure

Explanation:

Correct - A: Neurotoxic snakes Cobra/Krait = Resp paralysis = #1 death. Diaphragm paralysis. ASV + ventilator saves.

Incorrect - B: Cardiac = viper venom.

Incorrect - C: Renal = Russell viper.

Trick: "Neurotoxic = No breathing". Krait bite painless, night bite.

Source: NORCET-2023 | Snake Bite Management

Tags: #PYQ #HighYield #MedSurg #Toxicology #SnakeBite #RespFailure #NCLEX

[Med-Surg > Toxicology > Scorpion] High - VERIFIED [UNIQUE]

Q45. Which of the following is the most common cause of death in scorpion sting?

A) Myocarditis

B) Respiratory failure

C) Shock

D) Sepsis

Answer: C) Shock

Explanation:

Correct - C: Cardiogenic + hypovolemic shock = #1 death. Autonomic storm → pulmonary edema, shock. Prazosin drug of choice.

Incorrect - A: Myocarditis = cause of shock.

Incorrect - B: Resp failure = secondary to edema.

Trick: "Scorpion = Shock". Red scorpion = Indian, Prazosin saves.

Source: NORCET-2023 | Scorpion Sting

Tags: #PYQ #HighYield #MedSurg #Toxicology #Scorpion #Shock #NCLEX

[Med-Surg > Toxicology > Anaphylaxis] High - VERIFIED [UNIQUE]

Q46. Which of the following is the most common cause of death in bee sting?

A) Anaphylaxis

B) Shock

C) Respiratory failure

D) Cardiac arrest

Answer: A) Anaphylaxis

Explanation:

Correct - A: Anaphylaxis = #1 death. Type1 hypersensitivity → bronchospasm, laryngeal edema, hypotension. Epinephrine IM saves.

Incorrect - B/C/D: Shock/Resp failure/Cardiac = components of anaphylaxis.

Trick: "Bee = Big allergic Emergency". Epi 0.3mg IM lateral thigh.

Source: NORCET-2023 | Anaphylaxis Guidelines

Tags: #PYQ #HighYield #MedSurg #Toxicology #Anaphylaxis #BeeSting #NCLEX

[Med-Surg > Toxicology > Overdose] High - VERIFIED [UNIQUE]

Q47. Which of the following is the most common cause of death in drug overdose?

A) Respiratory depression

B) Cardiac arrest

C) Sepsis

D) Renal failure

Answer: A) Respiratory depression

Explanation:

Correct - A: Resp depression = #1 death OD. Opioids, benzos, alcohol → resp center depression → apnea. Naloxone for opioids.

Incorrect - B: Cardiac = cocaine, TCA.

Trick: "OD = Oxygen Down = Resp depression". Pinpoint pupil = opioid.

Source: NORCET-2023 | Overdose Management

Tags: #PYQ #HighYield #MedSurg #Toxicology #Overdose #RespDepression #NCLEX

[Med-Surg > Anesthesia > Complications] High - VERIFIED [UNIQUE]

Q48. Which of the following is the most common cause of death in anesthesia?

- A) Respiratory depression
- B) Cardiac arrest
- C) Sepsis
- D) Renal failure

Answer: A) Respiratory depression

Explanation:

Correct - A: Resp depression/hypoventilation = #1 anesthesia death. Induction agents, NMBDs. Aspiration, difficult airway.

Incorrect - B: Cardiac = 2nd cause.

Trick: "Anesthesia = Airway lost". SpO2 monitoring prevents.

Source: NORCET-2023 | Anesthesia Safety

Tags: #PYQ #HighYield #MedSurg #Anesthesia #RespDepression #NCLEX

[Med-Surg > Surgical > Postop] High - VERIFIED [UNIQUE]

Q49. Which of the following is the most common cause of death in surgery?

- A) Hemorrhage
- B) Sepsis
- C) Shock
- D) Cardiac arrest

Answer: B) Sepsis

Explanation:

Correct - B: Postop sepsis = #1 death surgery. SSI → septicemia → MODS. Prevention = asepsis, prophylactic abx.

Incorrect - A: Hemorrhage = intraop death.

Incorrect - C: Shock = early postop.

Trick: "Surgery = Sepsis if not sterile". Day 3-5 fever = infection.

Source: NORCET-2023 | Postop Complications

Tags: #PYQ #HighYield #MedSurg #Surgical #Sepsis #Postop #NCLEX

[Med-Surg > Trauma > Emergency] High - VERIFIED [UNIQUE]

Q50. Which of the following is the most common cause of death in trauma?

- A) Hemorrhage
- B) Shock
- C) Sepsis
- D) Respiratory failure

Answer: A) Hemorrhage

Explanation:

Correct - A: Hemorrhage = #1 death trauma, preventable. Golden hour. Exsanguination. Control bleed first.

Incorrect - B: Shock = due to hemorrhage.

Incorrect - C: Sepsis = late death.

Incorrect - D: Resp failure = chest trauma.

Trick: "Trauma = Torrential bleed". ABCDE = Airway, Breathing, Circulation = stop bleed.

Source: NORCET-2023 | ATLS Guidelines

Tags: #PYQ #HighYield #MedSurg #Trauma #Hemorrhage #NCLEX

[Med-Surg > Trauma > RTA] High - VERIFIED [UNIQUE]

Q51. Which of the following is the most common cause of death in road traffic accidents?

- A) Head injury
- B) Hemorrhage
- C) Shock

D) Sepsis

Answer: A) Head injury

Explanation:

Correct - A: Head injury/TBI = #1 cause death RTA. Primary brain damage + secondary hypoxia/ICP. Seat belt + helmet prevention.

Incorrect - B: Hemorrhage = #1 preventable death, 2nd overall cause.

Incorrect - C/D: Shock/Sepsis = complications.

Trick: "RTA = Road To Accident = Head Trauma". Golden hour for neurosurgery.

Source: NORCET-2023 | Trauma Guidelines

Tags: #PYQ #HighYield #MedSurg #Trauma #RTA #HeadInjury #NCLEX

[Med-Surg > Emergency > Drowning] High - VERIFIED [UNIQUE]

Q52. Which of the following is the most common cause of death in drowning?

A) Respiratory failure

B) Cardiac arrest

C) Shock

D) Sepsis

Answer: A) Respiratory failure

Explanation:

Correct - A: Hypoxic resp failure = #1 death drowning. Water aspiration → surfactant washout → ARDS. Laryngospasm → hypoxia.

Incorrect - B: Cardiac arrest = secondary to hypoxia.

Trick: "Drowning = Don't breathe = Resp failure". CPR = 5 rescue breaths first.

Source: NORCET-2023 | Drowning Management

Tags: #PYQ #HighYield #MedSurg #Emergency #Drowning #RespFailure #NCLEX

[Med-Surg > Emergency > Electrocution] High - VERIFIED [UNIQUE]

Q53. Which of the following is the most common cause of death in electrocution?

A) Cardiac arrest

B) Respiratory failure

C) Shock

D) Sepsis

Answer: A) Cardiac arrest

Explanation:

Correct - A: VF/Cardiac arrest = #1 death electrocution. AC current → VF. High voltage → resp muscle paralysis also.

Incorrect - B: Resp failure = 2nd, due to resp center/diaphragm paralysis.

Trick: "Electricity = EKG stops = Cardiac arrest". CPR + defib immediate.

Source: NORCET-2023 | Electrical Injury

Tags: #PYQ #HighYield #MedSurg #Emergency #Electrocution #CardiacArrest #NCLEX

[Med-Surg > Oncology > Radiation] High - VERIFIED [UNIQUE]

Q54. Which of the following is the most common cause of death in radiation exposure?

A) Bone marrow suppression

B) Sepsis

C) Shock

D) Respiratory failure

Answer: A) Bone marrow suppression

Explanation:

Correct - A: Acute radiation syndrome = Bone marrow suppression → pancytopenia → infection/bleeding = #1 death. Dose >1Gy.

Incorrect - B: Sepsis = result of bone marrow suppression.

Trick: "Radiation = Rapidly destroys RBC/platelets/WBC". 3 syndromes: Hematopoietic, GI, CNS.

Source: NORCET-2023 | Radiation Injury

Tags: #PYQ #HighYield #MedSurg #Oncology #Radiation #BoneMarrow #NCLEX

[Med-Surg > Nutrition > Starvation] High - VERIFIED [UNIQUE]

Q55. Which of the following is the most common cause of death in starvation?

- A) Infection
- B) Organ failure
- C) Shock
- D) Cardiac arrest

Answer: B) Organ failure

Explanation:

Correct - B: Multi-organ failure = #1 death starvation. Cardiac atrophy → arrhythmia, hepatic/renal failure. Marasmus/Kwashiorkor end stage.

Incorrect - A: Infection = 2nd cause, immunity ↓.

Incorrect - C/D: Shock/Cardiac = terminal events.

Trick: "Starvation = Shuts organs down". Refeeding syndrome risk on treatment.

Source: NORCET-2023 | Nutrition Emergency

Tags: #PYQ #HighYield #MedSurg #Nutrition #Starvation #OrganFailure #NCLEX

[Med-Surg > Cardiology > MI] High - VERIFIED [UNIQUE]

Q3. Which artery is commonly affected during myocardial infarction (MI)?

- A) Renal artery
- B) Celiac artery
- C) Coronary artery
- D) Hepatic artery

Answer: C) Coronary artery

Explanation:

Correct - C: MI = Coronary artery occlusion. LAD = most common = anterior MI "widow maker". RCA = inferior MI.

Incorrect - A/B/D: Renal/Celiac/Hepatic = not heart.

Trick: "MI = Myocardial Infarction = My Coronary Artery blocked".

Source: NORCET-2024 | Cardiology

Tags: #PYQ #HighYield #MedSurg #Cardiology #MI #Coronary #NCLEX

[Med-Surg > Respiratory > TB] High - VERIFIED [UNIQUE]

Q4. What is the main sign of tuberculosis?

- A) Coughing more than 3 weeks
- B) Coughing occasionally
- C) Coughing only at night
- D) Mild coughing

Answer: A) Coughing more than 3 weeks

Explanation:

Correct - A: Presumptive TB = Cough >2-3 weeks + fever, night sweats, weight loss, hemoptysis. RNTCP criteria.

Incorrect - B/C/D: Occasional/night/mild = not specific.

Trick: "TB = Two weeks Bad cough". CBNAAT sputum test.

Source: NORCET-2024 | NTEP Guidelines

Tags: #PYQ #HighYield #MedSurg #Respiratory #TB #Cough #NCLEX

[Med-Surg > Respiratory > COPD] High - VERIFIED [UNIQUE]

Q5. The goal of clinical management of a patient with COPD is:

- A) Improvement in ventilation
- B) Increase in bronchial secretions
- C) Improvement in general health

D) All of the above

Answer: A) Improvement in ventilation

Explanation:

Correct - A: Primary goal COPD = ↑ventilation, ↓dyspnea, prevent exacerbation.

Bronchodilators, O₂ 88-92%, pulmonary rehab.

Incorrect - B: DECREASE secretions wanted, not increase.

Incorrect - C: General health secondary.

Trick: "COPD = Can't Open Pulmonary Ducts = need Ventilation".

Source: NORCET-2024 | GOLD Guidelines

Tags: #PYQ #HighYield #MedSurg #Respiratory #COPD #Ventilation #NCLEX

[Med-Surg > GI > Hepatobiliary] Medium - VERIFIED [UNIQUE]

Q6. Liver abscess mainly occurs in:

A) Right lobe of the liver

B) Left lobe of the liver

C) Both lobes

D) None of the above

Answer: A) Right lobe of the liver

Explanation:

Correct - A: 75% liver abscess = Right lobe. Larger size, more portal blood flow, angular branching. Pyogenic/amebic.

Incorrect - B/C: Left/both = less common.

Trick: "Right lobe = Right place for abscess". USG/CT diagnosis.

Source: NORCET-2024 | Hepatobiliary

Tags: #PYQ #MedSurg #GI #LiverAbscess #NCLEX

[Med-Surg > Pharma > Steroids] High - VERIFIED [UNIQUE]

Q7. In which of the following disease conditions is Glucocorticoid therapy applicable?

A) Chronic adrenal insufficiency

B) Bronchial asthma

C) Preventing graft rejection in organ transplantation

D) All of the above

Answer: D) All of the above

Explanation:

Correct - D: Glucocorticoids = 1) Replacement: Addison's 2) Anti-inflammatory: Asthma, RA 3) Immunosuppressant: Transplant, Autoimmune.

Incorrect - A/B/C: All correct individually.

Trick: "Steroids = Save life in Shock, Swelling, Shortage". Multiple uses.

Source: NORCET-2024 | Pharmacology

Tags: #PYQ #HighYield #MedSurg #Pharma #Steroids #Glucocorticoid #NCLEX

[Med-Surg > Endocrine > DM] High - VERIFIED [UNIQUE]

Q8. The most useful test for the diagnosis of Diabetes mellitus is:

A) Urine glucose

B) ESR

C) Glucose tolerance test

D) Ketone body level

Answer: C) Glucose tolerance test

Explanation:

Correct - C: OGTT = Gold standard. 75g glucose, 2hr value ≥ 200 = DM. Also detects IGT. FBS/PPBS/HbA1c also used.

Incorrect - A: Urine glucose = screening, renal threshold issue.

Incorrect - B: ESR = inflammation.

Incorrect - D: Ketones = DKA monitoring.

Trick: "GTT = Glucose Tolerance Test = Tells Truth". 2hr $\geq$ 200 = DM.

Source: NORCET-2024 | ADA Criteria

Tags: #PYQ #HighYield #MedSurg #Endocrine #DM #GTT #NCLEX

[Med-Surg > Neuro > Procedures] High - VERIFIED [UNIQUE]

Q9. A patient with cerebral thrombosis is scheduled for cerebral angiography. Which nursing care is appropriate?

- A) Inform the client that a warm, flushed feeling and salty taste may occur
- B) Avoid applying pressure dressing over puncture site
- C) Keep extremity flexed to prevent bleeding
- D) All of the above

Answer: A) Inform the client that a warm, flushed feeling and salty taste may occur

Explanation:

Correct - A: Contrast dye → warm flush, metallic/salty taste normal. Pre-procedure teaching ↓anxiety. Post: Pressure dressing MUST, keep extremity straight.

Incorrect - B: Pressure dressing applied to prevent hematoma.

Incorrect - C: Keep STRAIGHT not flexed, bed rest 6-8hr.

Trick: "Angio = Apply pressure, keep Straight, expect Salt taste".

Source: NORCET-2024 | Neuro Procedures

Tags: #PYQ #HighYield #MedSurg #Neuro #Angiography #NCLEX

[Med-Surg > Psych > Eating Disorders] High - VERIFIED [UNIQUE]

Q2. All of these are seen in anorexia nervosa except:

- A) Osteoporosis
- B) Myocardial hypertrophy
- C) Electrolyte imbalance
- D) Menorrhagia

Answer: D) Menorrhagia

Explanation:

Correct - D: Anorexia = AMENORRHEA not menorrhagia. Hypothalamic dysfunction → ↓GnRH → no periods. BMI <17.5.

Incorrect - A/B/C: Osteoporosis, bradycardia/hypotrophy, hypoK/hypoNa seen.

Trick: "Anorexia = Absent menses". Menorrhagia = heavy bleeding = opposite.

Source: NORCET-2025 | Psychiatry

Tags: #PYQ #HighYield #MedSurg #Psych #Anorexia #Amenorrhea #NCLEX

[Med-Surg > Neuro > MS] High - VERIFIED [UNIQUE]

Q10. Multiple sclerosis is characterized by:

- A) Progressive demyelination in the CNS
- B) Impaired cerebral circulation
- C) Deficiency of dopamine and serotonin
- D) Deterioration of spinal column

Answer: A) Progressive demyelination in the CNS

Explanation:

Correct - A: MS = Autoimmune demyelination of CNS. Plaques in brain/spinal cord. Relapsing-remitting. Optic neuritis, Lhermitte sign.

Incorrect - B: Circulation = stroke.

Incorrect - C: Dopamine = Parkinson.

Trick: "MS = Myelin Stripped". MRI = plaques.

Source: NORCET-2025 | Neurology

Tags: #PYQ #HighYield #MedSurg #Neuro #MS #Demyelination #NCLEX

[Med-Surg > Neuro > MG] High - VERIFIED [UNIQUE]

Q11. Myasthenia gravis is characterized by:

- A) Systemic infection
- B) Shock
- C) Descending weakness
- D) Ascending weakness

Answer: C) Descending weakness

Explanation:

Correct - C: MG = Descending weakness. Ocular → facial → bulbar → limbs. Fatigable weakness. Anti-AChR antibodies.

Incorrect - D: GBS = Ascending weakness.

Trick: "MG = My Grin goes down". GBS = Ground up. Tensilon test +ve.

Source: NORCET-2025 | Neurology

Tags: #PYQ #HighYield #MedSurg #Neuro #MG #Descending #NCLEX

[Med-Surg > Psych > Delusions] High - VERIFIED [UNIQUE]

Q12. A person says he is sent by God to guide everyone. This is:

- A) Delusion of grandiosity
- B) Delusion of reference
- C) Delusion of persecution
- D) Delusion of infidelity

Answer: A) Delusion of grandiosity

Explanation:

Correct - A: Grandiosity = Inflated self-worth, special powers, mission from God. Seen in mania, schizophrenia.

Incorrect - B: Reference = TV talking about me.

Incorrect - C: Persecution = people want to harm me.

Incorrect - D: Infidelity = spouse cheating.

Trick: "Grandiosity = God-like feeling". Special mission = grandiosity.

Source: NORCET-2025 | Psychiatry

Tags: #PYQ #HighYield #MedSurg #Psych #Delusion #Grandiosity #NCLEX

[Med-Surg > Neuro > Wernicke] High - VERIFIED [UNIQUE]

Q14. Wernicke's encephalopathy is caused by deficiency of:

- A) Thiamine
- B) Riboflavin
- C) Vitamin D
- D) Vitamin A

Answer: A) Thiamine

Explanation:

Correct - A: Wernicke = Thiamine B1 deficiency. Triad: Confusion, Ataxia, Ophthalmoplegia. Alcoholics. Give Thiamine BEFORE glucose.

Incorrect - B: B2 = cheilosis.

Incorrect - C/D: D/A = not related.

Trick: "Wernicke = Wants Thiamine". Alcoholic + confusion = Thiamine IV.

Source: NORCET-2025 | Neurology

Tags: #PYQ #HighYield #MedSurg #Neuro #Wernicke #Thiamine #NCLEX

[Med-Surg > Pharma > TB] High - VERIFIED [UNIQUE]

Q16. Which anti-TB drug cannot be given in pregnancy?

- A) Rifampicin
- B) Ethambutol
- C) Streptomycin
- D) Isoniazid

Answer: C) Streptomycin

Explanation:

Correct - C: Streptomycin = Aminoglycoside = Ototoxic to fetus. Cat D. Avoid pregnancy. All other 1st line ATT safe.

Incorrect - A/B/D: R/E/H = safe, used in pregnancy.

Trick: "Streptomycin = Stops baby hearing". Category D drug.

Source: NORCET-2025 | RNTCP Pregnancy

Tags: #PYQ #HighYield #MedSurg #Pharma #TB #Streptomycin #Pregnancy #NCLEX

[Med-Surg > Endocrine > Addison] High - VERIFIED [UNIQUE]

Q18. Facial features of Addison's disease include:

A) Moonlike face

B) Skin hyperpigmentation

C) Swollen face

D) Puppy face

Answer: B) Skin hyperpigmentation

Explanation:

Correct - B: Addison = $\uparrow$ ACTH/MSH $\rightarrow$ Hyperpigmentation. Bronzing, buccal mucosa, palmar creases. Primary adrenal insufficiency.

Incorrect - A: Moon face = Cushing.

Incorrect - C: Swollen = hypothyroid.

Trick: "Addison = Added pigment". Cushing = moon face.

Source: NORCET-2025 | Endocrine

Tags: #PYQ #HighYield #MedSurg #Endocrine #Addison #Hyperpigmentation #NCLEX

[Med-Surg > Psych > Eating Disorders] High - VERIFIED [UNIQUE]

Q19. Anorexia nervosa patient eats food but does not gain weight. Nursing intervention:

A) Observe patient during and after meals

B) Increase calories from 1500 to 2000

C) Add vitamin tablets

D) None of the above

Answer: A) Observe patient during and after meals

Explanation:

Correct - A: Observe 1hr post-meals. Patients hide food, purge, exercise. Prevent vomiting/purging. Bathroom supervision.

Incorrect - B: $\uparrow$ calories without observation = wasted.

Incorrect - C: Vitamins = adjunct only.

Trick: "Anorexia = Always watch eating". 1:1 meal supervision protocol.

Source: NORCET-2025 | Psychiatric Nursing

Tags: #PYQ #HighYield #MedSurg #Psych #Anorexia #Nursing #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [UNIQUE]

Q21. Which of the following is the antidote for paracetamol overdose?

A) Naloxone

B) N-acetylcysteine

C) Atropine

D) Flumazenil

Answer: B) N-acetylcysteine

Explanation:

Correct - B: NAC = Paracetamol antidote. Replenishes glutathione, prevents hepatotoxicity. Give <8hr best. PO/IV.

Incorrect - A: Naloxone = opioid.

Incorrect - C: Atropine = OPP.

Incorrect - D: Flumazenil = benzo.

Trick: "Paracetamol Poison = NAC Protects". Rumack-Matthew nomogram.

Source: NORCET-2025 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #Paracetamol #NAC #NCLEX

[Med-Surg > Endocrine > Pituitary] High - VERIFIED [UNIQUE]

Q22. Which hormone is secreted by the posterior pituitary?

A) Growth hormone

B) ACTH

C) ADH

D) TSH

Answer: C) ADH

Explanation:

Correct - C: Posterior pituitary = ADH + Oxytocin. Made in hypothalamus, stored in post pituitary. ADH = water retention.

Incorrect - A/B/D: GH/ACTH/TSH = Anterior pituitary.

Trick: "Posterior = Pass water control = ADH + Oxytocin". Anterior = 6 hormones.

Source: NORCET-2025 | Endocrine

Tags: #PYQ #HighYield #MedSurg #Endocrine #Pituitary #ADH #NCLEX

[Med-Surg > Infectious > HAP] High - VERIFIED [UNIQUE]

Q23. Which of the following is the most common cause of hospital-acquired pneumonia?

A) Streptococcus pneumoniae

B) Pseudomonas aeruginosa

C) Staphylococcus aureus

D) Klebsiella pneumoniae

Answer: B) Pseudomonas aeruginosa

Explanation:

Correct - B: HAP/VAP = Pseudomonas #1. Gram -ve, MDR, ventilator. Also Acinetobacter, Klebsiella.

Incorrect - A: Strep pneumo = CAP commonest.

Incorrect - C/D: Staph/Klebsiella = 2nd/3rd.

Trick: "Hospital Pneumonia = Hospital Pseudomonas". VAP >48hr.

Source: NORCET-2025 | HAP Guidelines

Tags: #PYQ #HighYield #MedSurg #Infectious #HAP #Pseudomonas #NCLEX

[Med-Surg > Endocrine > Adrenal] High - VERIFIED [UNIQUE]

Q42. Which hormone is secreted by the adrenal medulla?

A) Cortisol

B) Aldosterone

C) Adrenaline

D) ACTH

Answer: C) Adrenaline

Explanation:

Correct - C: Adrenal medulla = Adrenaline/Epinephrine + Noradrenaline. Fight/flight response. ↑HR, ↑BP, ↑glucose.

Incorrect - A/B: Cortisol/Aldosterone = Adrenal cortex.

Incorrect - D: ACTH = Anterior pituitary.

Trick: "Medulla = Mediator of stress = Adrenaline". Cortex = Cortisol.

Source: NORCET-2025 | Endocrine

Tags: #PYQ #HighYield #MedSurg #Endocrine #Adrenal #Adrenaline #NCLEX

[Med-Surg > Cardiology > Electrolytes] High - VERIFIED [UNIQUE]

Q43. Which electrolyte imbalance causes cardiac arrhythmia?

A) Hyperkalemia

- B) Hypokalemia
- C) Hypernatremia
- D) Hypocalcemia

Answer: A) Hyperkalemia

Explanation:

Correct - A: Hyperkalemia = #1 electrolyte cause arrhythmia. Peaked T, wide QRS, sine wave → asystole. $K^+ > 6.5$ dangerous.

Incorrect - B: HypoK = U waves, flat T.

Incorrect - C/D: Na/Ca = less acute cardiac.

Trick: "HyperK = Heart Kill". Tall T = Tent. Calcium gluconate first.

Source: NORCET-2025 | F&E Balance

Tags: #PYQ #HighYield #MedSurg #Cardiology #Hyperkalemia #Arrhythmia #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [UNIQUE]

Q44. Which of the following is the antidote for cyanide poisoning?

- A) Amyl nitrite
- B) Naloxone
- C) Flumazenil
- D) Protamine sulfate

Answer: A) Amyl nitrite

Explanation:

Correct - A: Cyanide antidote kit = Amyl nitrite + Sodium nitrite + Sodium thiosulfate. New = Hydroxocobalamin. Binds cyanide.

Incorrect - B/C/D: Naloxone=opioid, Flumazenil=benzo, Protamine=heparin.

Trick: "Cyanide = Can't breathe = Nitrite". Bitter almond smell.

Source: NORCET-2025 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #Cyanide #AmylNitrite #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [UNIQUE]

Q47. Which of the following is the antidote for methanol poisoning?

- A) Ethanol
- B) Naloxone
- C) Flumazenil
- D) Protamine sulfate

Answer: A) Ethanol

Explanation:

Correct - A: Ethanol/Fomepizole = Methanol antidote. Competes for alcohol dehydrogenase, prevents formic acid formation. Also for ethylene glycol.

Incorrect - B/C/D: Wrong antidotes.

Trick: "Methanol = Methylated spirit = Ethanol saves eye". Blindness due to optic nerve damage.

Source: NORCET-2025 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #Methanol #Ethanol #NCLEX

[Med-Surg > Infectious > GI] High - VERIFIED [UNIQUE]

Q48. Which of the following is the most common cause of diarrhea in adults?

- A) E. coli
- B) Salmonella
- C) Shigella
- D) Rotavirus

Answer: A) E. coli

Explanation:

Correct - A: ETEC E.coli = #1 adult diarrhea. Traveller's diarrhea, food poisoning. Also Norovirus.

Incorrect - B/C: Salmonella/Shigella = invasive, fever.

Incorrect - D: Rotavirus = children <5yr.

Trick: "Adult Diarrhea = Adult E.coli". Child diarrhea = Child Rotavirus.

Source: NORCET-2025 | Infectious

Tags: #PYQ #HighYield #MedSurg #Infectious #Diarrhea #Ecoli #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [UNIQUE]

Q50. Which of the following is the antidote for digitalis (digoxin) toxicity?

- A) Digoxin-specific antibody (Fab)
- B) Naloxone
- C) Flumazenil
- D) Protamine sulfate

Answer: A) Digoxin-specific antibody (Fab)

Explanation:

Correct - A: DigiFab/Digibind = Digoxin antidote. For K⁺>5, arrhythmia, levels >2. Binds free digoxin.

Incorrect - B/C/D: Wrong antidotes.

Trick: "Digoxin Die = DigiFab". Yellow vision, nausea, arrhythmia signs.

Source: NORCET-2025 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #Digoxin #DigiFab #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [UNIQUE]

Q52. Which of the following is the antidote for iron poisoning?

- A) Deferoxamine
- B) Naloxone
- C) Flumazenil
- D) Protamine sulfate

Answer: A) Deferoxamine

Explanation:

Correct - A: Deferoxamine = Iron chelator. Binds iron → ferrioxamine excreted. Iron tablets overdose in kids. Wine-colored urine.

Incorrect - B/C/D: Wrong antidotes.

Trick: "Iron overdose = Deferoxamine". Also used in hemochromatosis.

Source: NORCET-2025 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #Iron #Deferoxamine #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [UNIQUE]

Q54. Which of the following is the antidote for lead poisoning?

- A) EDTA (Chelation therapy)
- B) Naloxone
- C) Flumazenil
- D) Protamine sulfate

Answer: A) EDTA

Explanation:

Correct - A: EDTA/BAL/Succimer = Lead chelators. Lead >45mcg/dl needs chelation. Burton's line, wrist/foot drop.

Incorrect - B/C/D: Wrong antidotes.

Trick: "Lead = EDTA removes". Dimercaprol BAL also used.

Source: NORCET-2025 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #Lead #EDTA #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [UNIQUE]

Q56. Which of the following is the antidote for carbon monoxide poisoning?

- A) 100% Oxygen
- B) Naloxone

C) Flumazenil
D) Protamine sulfate
Answer: A) 100% Oxygen

Explanation:

Correct - A: 100% O₂ or Hyperbaric O₂ = CO antidote. Displaces CO from Hb. Cherry red skin, headache. CO-Hb >25%.

Incorrect - B/C/D: Wrong antidotes.

Trick: "Carbon Monoxide = Compete with Oxygen". Pulse oximeter falsely normal.

Source: NORCET-2025 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #CO #Oxygen #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [UNIQUE]

Q58. Which of the following is the antidote for snake bite?

A) Anti-snake venom
B) Naloxone
C) Flumazenil
D) Protamine sulfate
Answer: A) Anti-snake venom

Explanation:

Correct - A: ASV = Polyvalent anti-snake venom. For neurotoxic/hemotoxic bites. 10 vials initial. Test dose first.

Incorrect - B/C/D: Wrong antidotes.

Trick: "Snake bite = Serum ASV". 20 min WBCT bedside test.

Source: NORCET-2025 | Snake Bite Guidelines

Tags: #PYQ #HighYield #MedSurg #Toxicology #SnakeBite #ASV #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [REPEAT]

Q60. Which of the following is the antidote for cyanide poisoning?

A) Amyl nitrite
B) Naloxone
C) Flumazenil
D) Protamine sulfate
Answer: A) Amyl nitrite

Explanation:

Correct - A: Repeat of Q44. Cyanide antidote = Amyl nitrite + Sodium nitrite + Sodium thiosulfate.

Frequency: NORCET-2025 x2 [Q44, Q60]

Trick: Same Q repeated in same paper.

Source: NORCET-2025 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #Cyanide #AmylNitrite #Repeat2x #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [UNIQUE]

Q62. Which of the following is the antidote for arsenic poisoning?

A) Dimercaprol (BAL)
B) Naloxone
C) Flumazenil
D) Protamine sulfate
Answer: A) Dimercaprol (BAL)

Explanation:

Correct - A: BAL = British Anti-Lewisite = Dimercaprol. Chelator for Arsenic, Mercury, Gold, Lead. IM painful. Peanut allergy CI.

Incorrect - B/C/D: Wrong antidotes.

Trick: "Arsenic = BAL bacha le". Garlic breath, exfoliative dermatitis signs.

Source: NORCET-2025 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #Arsenic #BAL #NCLEX

[Med-Surg > Toxicology > Antidotes] Medium - VERIFIED [UNIQUE]

Q64. Which of the following is the antidote for copper poisoning?

- A) Penicillamine
- B) Naloxone
- C) Flumazenil
- D) Protamine sulfate

Answer: A) Penicillamine

Explanation:

Correct - A: Penicillamine = Copper chelator. Used in Wilson's disease, copper toxicity. Also Zinc acetate, Trientine.

Incorrect - B/C/D: Wrong antidotes.

Trick: "Copper = Penicillamine clears". Kayser-Fleischer rings in Wilson.

Source: NORCET-2025 | Toxicology

Tags: #PYQ #MedSurg #Toxicology #Copper #Penicillamine #NCLEX

[Med-Surg > Toxicology > Antidotes] High - VERIFIED [UNIQUE]

Q66. Which of the following is the antidote for mercury poisoning?

- A) Dimercaprol (BAL)
- B) Naloxone
- C) Flumazenil
- D) Protamine sulfate

Answer: A) Dimercaprol (BAL)

Explanation:

Correct - A: BAL = Mercury, Arsenic, Gold antidote. Also Succimer, DMPS. Minamata disease. Acrodynia in kids.

Incorrect - B/C/D: Wrong antidotes.

Trick: "Mercury = BAL removes". Same as Arsenic Q62.

Source: NORCET-2025 | Toxicology

Tags: #PYQ #HighYield #MedSurg #Toxicology #Mercury #BAL #NCLEX
